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Stakeholder experiences of the response to community-based mental health crisis: A qualitative systematic review.

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Abstract

Objective: To review and synthesise qualitative studies that have explored perspectives of stakeholders (people with lived experience of mental illness, their families and first responders) of the interactions between first responders (FRs) and people who experience a mental health crisis (MHC) in the community.
Design: A systematic review of qualitative evidence was conducted. English language articles exploring the content of interactions and participants' experiences were included.
Data sources: Databases and websites searched for relevant literature: MEDLINE, PsycINFO, EMBASE, CINAHL; Google scholar, SAGE journals, Science Direct, and PubMed.
Data extraction and synthesis: Two reviewers read and systematically extracted data from the included papers. Papers were appraised for methodological rigour using the Critical Appraisal Skills Programme (CASP) Qualitative Checklist. Data were thematically analysed.
Results: We identified 3483 unique records, 404 full-texts were assessed against the inclusion criteria and 79 studies (3 unpublished) were included in the qualitative synthesis. FRs identified in studies were police and ambulance staff. Findings were organised into 4 areas: Criminalising versus recognising MHC: Legitimacy and stigma; Experience and skills in dealing with MHC; Impact on family/carers and how their involvement may hinder or facilitate FR response. FRs personal experience of mental illness and focused training can help create a more empathetic response, however lack of resources in mental health services continues to be a barrier where 'frequent attenders' are repeatedly let down by MH services.
Conclusion: The lack of resources in mental healthcare and rise in mental illness, suggests that FR response to MHC is inevitable. Inconsistent training, complexity of procedures and persistent stigmatisation make this a very challenging task. Improving communication with family carers and colleagues can make a difference. Broader issues of legitimacy and procedural barriers should be considered in order to reduce criminalisation and ensure an empathetic response.

Strengths and limitations of this study

- As far as we are aware, this is the first systematic review bringing together the experiences of all stakeholders involved in a mental health crisis in the community.
- We undertook rigorous searches for the available literature and assessed the quality of the studies.
- Sixty-six of the included studies were assessed to be of 'high' methodological quality (≥9 out of 10) and 13 were of 'intermediate' quality (7 or 8 out of 10).
- The majority of studies were conducted in developed countries, which could limit the generalisability of the findings.
- This review included studies published in the English language only.

Background

Mental health-related crises (MHC) in the community are increasing. This can partly be attributed to increased transitions from institutions to community treatment,[1] while the Covid-19 pandemic also had an impact, with the NHS reporting a significant increase in urgent and emergency referrals of people in crisis.[2] Police and paramedics are often the first responders (FRs) to a MHC.[3,4] In the UK 40% of police time is spent responding to a MHC.[5] The Department of Health[6] developed guidance which recommends that when people experience a MHC where the police responds, should expect 'appropriate help' and a 'prompt, efficient and respectful' process.

However, there are concerns that FRs cannot provide expert support for people with mental illness (PMI) and report feeling ill-equipped.[7, 8] Limited training can result in inappropriate responses, [5, 9] for instance, tasers are more likely to be used during a MHC than a criminal arrest.5 Due to limited resources the only options available to police have been force and detention.[9] Section 136 (S136) of the Mental Health Act 1983 gives police authority to detain PMI in a safe place, often a cell, which can worsen the crisis.[9,10] Concerns about police capability to make decisions about people's mental health has led to changes in legislation and policy. The Policing and Crime Act 2017 requires that police consult mental health professionals (MHPs) before using S136, use a health-based place of safety instead of a police station, and reduce the period of detention from 72 to 24 hours.[11]

Ways to improve FR response include specific training tools[12] which mainly involving de-escalation techniques for police, as well as models of triage[7] that aim to improve police officers' ability to provide effective response.[13] Triage includes Crisis Intervention Teams (CIT) where a police member is trained in mental health crisis intervention, and co-response/interagency collaborations between FR and mental health professionals[7]. The aim is to improve response and to reduce the likelihood of detention.[7] A recent review[13] of interagency collaboration models found that there is a wide range, with different kinds of agencies and services involved at various levels. Co-responder interventions are not routine practice and the majority not evaluated,[13] and high costs, logistics[14,13] and limited access to local MH services[15] make such models difficult to implement.

Currently, evidence on successful communication strategies for emergency services is not well documented nor integrated into routine practice.[11] Factors that impact on the interactions between FRs and PMI need to be understood better and integrated into training to improve response.[16] Significant research focuses on service development, however there is a need for research on lived experiences and interactions as they unfold in real time, exploring the perspectives of all stakeholders.[17, 18] With rises in mental distress and illness worldwide, identifying experiences of the responses to MHC from all people involved is critical. Hence, the aim of this study was to review available qualitative evidence of the perspectives of stakeholders (people with lived experience, their companions and first responders) of the interaction/communication of the FR response to mental health crises in the community.

Methods

The PICo for qualitative studies (figure 1) was used to identify the *Population* of interest, the phenomena of *Interest* and the *Context*.[19]

Figure 1. PICo

Search Strategy

A systematic search was conducted using the MEDLINE in Process (Ovid), PsycINFO (Ovid), EMBASE (Ovid) and CINAHL databases. Relevant research was also identified using Google scholar, SAGE journals, Science Direct, PubMed, Hand searching systematic reviews' reference lists, reference checking and citation chasing of included studies. A search strategy was developed using

controlled vocabulary unique to each database and free-term texts including three search groups: Mental health crisis; non-mental health emergency responders, and qualitative design (appendix 1).

Selection criteria

Retrieved articles from the inception for each database to November 2020 were included, if they directly related to all three search groups. Articles published in English were included because there was no multilingual researcher in the team, and finite resources. Studies reporting perspectives/experiences, involving adults (≥18 years) and using a qualitative approach, including mixed methods with a clear qualitative aspect. We only included qualitative articles to allow for understanding of the experiences of all stakeholders involved in these interactions. Studies with exclusively quantitative data, non-adult participants, that reported solely on mental health professionals, and the MHC did not occur in the community, were excluded.

Screening, data extraction and synthesis

Two reviewers conducted an initial screening of all titles and abstracts against the inclusion criteria to identify relevant papers. Duplicates were removed. Retrieved records were first screened on title and abstract. Reviewers screened the relevant full papers, and studies were independently assessed for eligibility. Disparities were resolved in weekly meetings before proceeding to the next stage. The Critical Appraisal Skills Programme (CASP) Qualitative Checklist[20] was used to appraise the methodological quality of the articles. The extraction and analysis of the qualitative studies was iterative[21] and was reviewed in weekly analytic meetings. Findings of each study, including participant quotations and author interpretations, were examined, extracted, and assigned to descriptive codes in NVIVO12. Thematic synthesis was used to code the findings of primary studies and develop descriptive themes.

Patient and Public Involvement

Exploratory work with police staff in 2018, paramedics and PMI was conducted which led to development of this study:

1. Carers and people with lived experience (South West of England)
2. Ambulance Service: 4 paramedics and one Patient Engagement Manager
3. Devon & Cornwall Police lead and for Enhanced Communication

Results

The Electronic and hand searches identified 4297 studies (Figure 2, PRISMA flow diagram). Seventy-nine studies, including 3 unpublished (22-24), were included in the review (Table 1). These studies used semi-structured qualitative interviews (n=43), focus groups (n=4) and mixed methods (n=32). Studies explored the perspectives of the police (n=35), paramedics (n=13), PMI (n=15), carers or family members (n=8) and multiple stakeholders (n=8). No other FR perspectives, e.g. firefighters, were found. The studies were conducted in the UK (n=22), USA (n=22), Australia (n=15), Canada (n=10), The Netherlands (n=2), New Zealand, France, Sweden, Ireland, Ghana, Slovenia, Portugal, and Guyana (each n=1). All scored 8 or higher on the CASP Checklist, indicating medium (not stating whether the relationship between researchers and participants was considered) to high quality; no exclusions were made on this basis.

Figure 2. PRISMA Flow Diagram of included and excluded studies.

Table 1. Summary of included studies.

Author & year	Country	Perspective	Study focus	Study design and methods	Sample size (qualitative)
Adio *et al* 2020[92]	USA	Paramedics	Views of paramedics towards frequent emergency department users and underlying causes.	Mixed-method design. Survey & interviews.	16 Community Integrated Health Program paramedics.

Baker and Pillinger 2020[48]	USA	Family members	To understand how police interactions with PMIs can lead to deaths and families' experiences.	Semi-structured interviews.	16 family members of a citizen who died after police contact.
Baker and Pillinger 2020[51]	UK	Family members	The police role in providing a de facto service for people undergoing mental health crises.	Semi-structured interviews.	12 family members of a citizen who died after police contact.
Bendelow *et al* 2019[54]	UK	PMI & companions	To investigate the complexities underlying the high rates of s136.	Mixed-methods. Interviews & written accounts.	37 people with lived experience of detention.
Bohrman *et al* 2018[84]	USA	Police	Police officers assessment for mental illnesses and variation by location.	Semi-structured interviews.	15 officers.
Boscarato *et al* 2014[29]	Australia	PMI	Consumers' experiences with formal crisis-response mechanisms - police-only and joint responses.	Exploratory study. Semi-structured interviews.	11 mental health consumers.
Bradbury *et al* 2017[32]	Australia	PMI, carers & Service providers	To explore the lived experience of involuntary transport under the mental health act.	Cross-sectional, qualitative research design. Interviews.	16 participants: 6 consumers, 4 carers 6 service providers.
Brennan *et al* 2016[44]	Australia	Carers	Carers' experiences of mental health crises and responses provided by police and mental health services.	Semi-structured interviews.	9 carers for someone with a mental illness.
Brink *et al* 2011[35]	Canada	PMI	How people with mental illness perceive and interact with the police.	Participatory Action Research. Focus groups, interview, survey.	60 people with lived experience.
Callender *et al* 2021[17]	UK	Police	To compare the processes, experiences and perceptions of Mental health street triage.	Semi-structured interviews.	27 police and health service staff.
Canada *et al* 2012[58]	USA	Police	Impact of CIT on police officers' response to mental illness calls.	Qualitative research design. Survey and phone interviews.	216 officers.
Cheng *et al* 2020[93]	USA	Family caregivers	Needs of family caregivers for early psychosis.	Semi-structured focus group interviews.	20 family caregivers.
Copeland & Heilemann 2008[25]	USA	Family caregivers	Mothers understanding of violence of their mentally ill, adult children.	Grounded theory methodology, open-ended interviews.	8 mothers.
Booty *et al* 2020[89]	USA	Police	Officer attitudes pre- and post-CIT training.	Mixed-methods study. Surveys, group discussions, interviews.	14 Central District patrol officers.
Daggenvoorde *et al* 2018[30]	Netherlands	PMI & families	Lived experiences of patients with a psychotic or bipolar disorder and their families of a mobile crisis team.	Phenomenological study design. Interviews.	10 patients and10 family members.
Davey *et al* 2019[87]	New Zealand	Police	E-Learning for police with regard to people experiencing mental distress.	Semi-structured telephone interviews.	24 police staff.
Dejean *et al* 2016[94]	Canada	Paramedics	Paramedics views on appropriate versus inappropriate ambulance use.	Constructivist grounded theory methodology. Interviews.	19 paramedics.

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Dougall 2014[22]	Canada	Police	Lived experiences of frontline police personnel of a mid-sized police service.	Exploratory study, heuristic phenomenology. Semi-structured interviews	7 police officers, 4 communicators and 3 administrators.
Dyer *et al* 2015[91]	UK	Police	Implementation of Cleveland Police's pilot Street Triage (ST) service.	Exploratory design. Interviews and retrospective analysis of case notes.	16 stakeholders, case notes, and administrative data.
Erdner & Piskator 2013[57]	Sweden	Police	Police officers' experiences of committing individuals with mental illness to the hospital for treatment.	Semi-structured interviews. Qualitative content analysis.	7 police officers.
Ferguson *et al* 2019[36]	Australia	PMI	Men's experiences of ambulance care for mental health and/or AOD problems.	Exploratory qualitative approach. Semi-structured interviews.	30 men.
Ford-Jones 2020[24]	Canada	Paramedics	Mental health and psychosocial calls encountered by paramedics in the community.	Case study. Interviews.	5 paramedic service management, 5 educators and 5 directors/physicians.
Ford-Jones & Daly 2020[95]	Canada	Paramedics	Paramedic service practices that are connecting patients to mental health and psychosocial programming.	Qualitative interviews and observations.	31 paramedics, 5 paramedic service management, 5 educators, 5 directors/physicians.
Gerson *et al* 2009[45]	USA	Family members	Experiences of families seeking treatment for young people with recent-onset psychosis.	Qualitative open-ended interviews.	14 particpants-13 interviews (one pair)
Gibbs & Haas 2020[33]	Australia	PMI	Experiences of autistic people who had interacted with police.	Mixed-methods design. Questionnaire and interviews.	12 autistic adults
Girard *et al* 2014[85]	France	PMI & Police	Mental health outreach team and the police department responding to the clinical needs of persons who are homeless with serious psychiatric disorders.	Mixed qualitative and quantitative methods. Focus group. Minutes of meetings.	40 PMI (interactions). Focus groups: 12 police officers.
Godfredson *et al* 2011[75]	Australia	Police	Contact between the police and people experiencing mental illness.	Survey. Thematic analyses of open-ended responses.	3,534 police officers.
Goodall *et al* 2019[37]	UK	PMI	Process of being detained under S136 of the Mental Health Act (1983, 2007).	Semi-structured interviews.	15 people detained under Section 136 of the Mental Health Act.
Green 1997[81]	USA	Police	Police involvement with suspects who have a mental illness.	Mixed methods. Qualitative structured & semi-structured interviews.	11 police officers
Gregory & Thompson 2013[39]	UK	PMI	Personal experiences of a service user through a mental health crisis.	Auto-ethnography and interviews. Reflective narrative.	1 service user.
Haas & Gibbs 2020[50]	Australia	PMI and parents	Impact that autistic characteristics on interactions with police.	Interviews, content analysis.	12 autistic adults and 19 parents.
Hanafi *et al* 2008[90]	USA	Police	To evaluate the effectiveness of Crisis Intervention Team (CIT) training for police officers.	Qualitative focus group study.	25 officers.

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Hassell 2020[72]	USA	Police	Efficacy of CIT Training	Observation and semi-structured interviews.	32 patrol officers; 27 providers/practitioners; 10 executives, CIT coordinators and UPD personnel.
Herrington & Pope 2014[38]	Australia	Police, NSW Health & NGO representatives	Evaluate the success of the MHIT.	A multi-phased, mixed methods design. Interviews, observations	56 interview participants.
Holmes 2017[26]	UK	PMI	Missing people with mental health issues and police response.	Qualitative semi-structured interview & free text narratives.	45 formerly missing adults with mental health issues.
Keefe *et al* 2020[60]	USA	Paramedics	Paramedics' experiences and perceptions regarding behavioral health emergencies.	Qualitative in-depth interviews.	25 paramedics.
Krayer *et al* 2018[96]	UK	Police; statutory & third sector organisations	Perceptions about joint working between mental health, social care and police services with regard to anti-social behaviour.	Multi-method sequential qualitative study. Police logs; interviews; focus groups.	55 statutory and third sector organisations; 60 cases-narrative police logs
Lamb & Tarpey 2019[61]	UK	Police	UK police officers views on their experience of working with people with mental health difficulties.	Social constructionist perspective. Interviews.	10 police officers
Lane 2019[53]	UK	Police	Forum members discourses on individuals experiencing mental health difficulties.	Discursive psychology and critical discourse analysis.	75 threads – discussion forum for members of the police force
Livingston *et al* 2014[55]	Canada	PMI	Experiences of people with mental illness of their interactions with the police.	Semi-structured interviews.	60 people with mental illness.
Mahmuda *et al* 2020[97]	Canada	PMI (Frequent users)	Frequent users' experiences regarding EMS.	A Grounded theory approach. Semi-structured interviews.	10 participants.
Marsden *et al* 2020[68]	UK	Police	UK police officers' views and experiences of their involvement in mental healthcare, focusing on decision-making in emergency situations.	Qualitative methods. Semi-structured interviews, including a vignette.	15 police officers.
McCann *et al* 2018[73]	Australia	Paramedic	Paramedics' experience of caring for patients with non-medical emergency- related mental health and/or AOD problems.	Framework Method. Semi-structured interviews.	73 paramedics.
McGuinness *et al* 2013[40]	Ireland	PMI	Impact of involuntary hospital admission to a mental health centre and illuminate their lived experience of involuntary admission.	Interpretative Phenomenological Analysis. Semi-structured interviews.	6 participants.
McKenna *et al* 2015[49]	Australia	Police, carers, paramedics, MH staff	Perceptions of stakeholders on behavioural escalation service utilization of people in mental health crisis.	Exploratory research design. Interviews	17 people (advisors, ED staff, police, ambulance officers).
McLean & Marshall 2010[76]	UK	Police	Challenges when policing mental ill-health and use of mental health triage in England and Wales.	Interpretative phenomenological analysis. Semi-structured interviews.	9 police officers.

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Morgan & Paterson 2019[59]	UK	Police	Police officers views of working with people with mental health difficulties and their experience of training to equip them for this.	Human rights framework. Semi-structured interviews.	7 police officers.
Myers *et al* 2019[46]	USA	PMI	Real-time decision-making between first hospital admission for early psychosis and treatment engagement or drop-out.	Prospective, longitudinal, ethnographic study. Open-ended, person-centred interviews	37 people who experienced involuntary admission.
Ogloff *et al* 2013[77]	Australia	Police	Association among mental disorder, police shootings, and other injuries, and police interactions with victims of crime.	Mixed-methods. semi-structured interviews.	25 police officers.
Olasoji *et al* 2017[74]	Australia	Carers	Experiences of carers as they access mental health services and as they engage with service/health providers?	A qualitative descriptive approach. focus groups	19 Carers of patients (i.e., family members)
Osafo *et al* 2017[62]	Ghana	Police	Views of the police on persons who attempt suicide and the law criminalizing the act.	Qualitative approach. Semi-structured in-depth interviews	18 officers
Oxburgh *et al* 2016[23]	UK	Police	Perceptions of police officers regarding MD suspects, support provided to MD suspects, and current police training in MD.	Grounded Theory. Questionnaire consisting of a mixture of open and probing questions	35 police officers
Prener & Lincoln 2015[98]	USA	Paramedic	Experiences, beliefs, and attitudes toward what EMS providers call “psych calls,”	Semi-structured observational method and interviews.	4 x12hr observations Interviews: 4 EMTs & 16 paramedics.
Railey *et al* 2020[79]	USA	Police , carer and PMI	LEOs’ knowledge of ASD, interactions between LEOs and individuals with ASD, and training needs.	Constructivist grounded theory approach (interviews and questionnaires).	6 adults with ASD, 5 caregivers and 6 law enforcement officers
Rant & Bregar 2014[56]	Slovenia	Paramedic	Paramedics experience suicidal patients.	Qualitative case study, semi-structured interviews.	10 paramedics.
Rees *et al* 2017[69] AND Rees *et al* 2018[67]	UK	Paramedic	Paramedics' perceptions and experiences of caring for people who self-harm.	Evolved Grounded Theory Methodology. Semi structured interviews.	11 paramedics.
Roberts & Henderson 2009[88]	Australia	Paramedic	Paramedics use of resources to support practice and their role when dealing with patients displaying mental illness.	Mixed methods. Focus groups and survey (open questions).	74 survey responders. Focus groups: 20 paramedics.
Rolfe *et al* 2020[63]	UK	Paramedic	Paramedics managing patients experiencing mental health issues.	Qualitative. Observations and interviews.	21 paramedics and 20 patients with mental illness.
Ross *et al* 2020[70]	Australia	Police	Effectiveness of the Masterplan in reducing suicides.	Mixed-methods design. Interviews.	8 police officers.
Salerno & Schuller 2019[66]	Canada	Police	Interactions between adults with ASD and the police and (2) exploring how individuals with ASD perceive these experiences.	Community-engaged research strategy and integrating both qualitative and	35 adults with ASD.

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				quantitative research methods.	
Schulenberg 2016[64]	Canada	Police	Dynamics of police and PMI encounters vs non-PMI.	Observation and sequential mixed methods research design. Field notes.	Data on 606 citizens (74 ride-alongs).
Shaban 2011[71]	Australia	Paramedics	Paramedics accounting for their JDM with respect to mental illness in the emergency care setting.	Ethnographic methods. Semi-structured interviews.	6 paramedics.
Soares & Pinto da Costa 2019[34]	Portugal	Police	Police officers' interactions in compulsory admissions and in the delivery of involuntary mental health treatment.	Qualitative research. In-depth semi-structured interviews.	10 police officers.
Sondhi *et al* 2018[41]	UK	PMI	Perceptions of the process for people who have been detained under section 136 of the Mental Health Act 1983.	Qualitative design. Semi-structured interviews.	58 people with lived experience detained under section 136 and 4 carers.
Spence & Millott 2016[31]	UK	Police	To explore police negotiators' perceptions of preparation and coping responses related to suicide.	Qualitative design. Semi-structured interviews.	16 police negotiators.
Stokoe & Sikveland 2020[99]	UK	Police	Methods negotiators use to engage persons in crisis and initiate and maintain productive sequences of talk.	Qualitative design. Conversation analysis.	14 negotiations, 31hrs of audio-recordings.
Tully & Smith 2015[86]	USA	Police	Officer perception of preparedness after receiving Crisis Intervention Team training.	Mixed-method design. semi-structured interviews	8 officers.
van Steden 2020[82]	Netherlands	Police	Officers and nurses perceptions of disorderly and confused people.	Qualitative design. informal conversations, interviews, focus groups, group meetings.	5 policy makers, 17 community police officers and 3 district nurses.
Wallace 2019[27]	Guyana	Police	Police officers' attitude, response and interaction with people with mental illness.	Mixed-method design. Open-ended questions of survey.	9 participants responded to open-ended questions.
Warrington 2019[42]	UK	People with suicidality	Repeated Section 136 detentions.	Mixed-method design. In-depth interviews	6 participants.
Watson *et al* 2008[52]	USA	PMI	Dimensions of how police encounters are experienced by consumers.	Qualitative design. In-depth semi-structured interviews	20 persons with mental illness.
Watson & Wood 2017[65]	USA	Police	Examination of the Crisis Intervention Team model.	Mixed-method design. In-depth semi-structured interviews.	21 police officers.
Wells & Schafer 2006[83]	USA	Police	Police officer perceptions of their contacts with the mentally ill and examine outcomes of an innovative police training program.	Mixed-method design. Open-ended survey items.	126 police officer respondents.
White 2019[43]	UK	PMI	Experiences with law enforcement services,	Author personal experience- narrative.	One personal account.

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			emergency departments and psychiatric services.		
Wood & Beierschmitt 2014[80]	USA	Police	Opportunities for police enhanced upstream engagement.	Mixed-method design. Focus groups & interviews.	22 police officers.
Wood *et al* 2020[47]	USA	Police	Officer perspectives on the unmet needs of individuals and their families and the ways in which the mental health and social system environment constrain officers' abilities to be responsive to them.	Qualitative design. Field notes describing obse rvations and ride-along interviews.	57 officers.
Wood & Watson 2017[28]	USA	Police	The nature of encounters police have with persons affected by mental illness and the ways in which such encounters are resolved by police.	Qualitative design. Observations.	31 ride-alongs. 53 officers observed.
Young *et al* 2019[78]	Canada	Parent Caregiver	Experiences of parent caregivers for adult children with schizophrenia.	Qualitative design. Interviews.	12 parent participants

Table 2 presents the themes and subthemes from the thematic synthesis of the included studies, which all contained first-order themes.

Theme	Subthemes	Studies exploring each sub-theme
Acknowledging versus criminalising mental illness	Force and physical restraint: criminalisation vs recognition	22, 24, 25-28, 29, 30-66,
	Accountability, cautiousness and confusions about the law and procedures.	17, 67, 31, 34, 59, 68-73
Legitimacy and stigma	Non-legitimacy of mental illness and impact on seeking help.	22, 24, 25, 28-32, 36, 42-44, 46-49, 53, 56, 60, 61, 67, 68, 74-77
	Stigma and use of stereotypes.	17, 23, 24, 25-28, 30, 31, 33-37, 39, 40, 47, 48, 50-57, 59-61, 63-67, 69, 71, 73, 75, 78-85
Experience and skills in dealing with a mental health crises	Training: impact and requirements.	22, 24, 27, 29, 31, 32, 35, 38, 52, 53, 55 , 57, 58, 60, 61, 63, 67-69, 86, 71-84, 87-90.
	Interagency collaboration.	17, 22, 24, 27, 29, 30, 32, 35, 37, 54, 55, 57, 59, 61, 76, 85, 87, 89, 91
	The value of personal experience.	22, 24, 28, 34, 60, 68, 70, 73, 77
Impact of response on companions: involvement hindering vs facilitating response	Companions ignored, not informed and impact on them not recognised.	24, 25, 26, 30-32, 35- 46, 74
	Companion involvement: hindering response vs leading to better outcomes.	24, 25-34, 67

Table 2. Themes and subthemes.

1. Acknowledging versus criminalising mental illness and procedural concerns.

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1.1 Force and physical restraint: criminalisation vs recognition

Companions and PMI described distress, fear, confusion, humiliation and trauma caused by a large police presence, disproportionate use of S136, involuntary transport, and physical restraint:[29, 32, 35-38, 40, 42-46, 48-55] *"You're confined in this tiny, small box and you go, 'what's happening to you?' They don't speak to you, and the only time they do, they're barking orders at you"* (PMI).[38] FRs were sometimes perceived as threatening:[29, 32, 35, 37, 43, 52, 53] *"I said I am going to call 911, and he said: 'no don't, if you call 911 they are going to kill me"* (companion).[48] In one study, paramedics describe disliking physical restraint and excessive force, believing their presence alone can be calming.[56] In contrast, police described their "right to use force",[57] arguing that force and restraint were *"most helpful because they immediately make the situation unpleasant for the individual"*.[58] Police felt victimized when criticized for using force.[34, 53, 58, 59]

PMI and their family/carers described instances where crises were not resolved. Time delays, use of force and FRs' threatening presence increased their distress:[29, 35-37, 38, 43, 46, 52] *"[Six police officers] were standing there with their capsicum spray…I was just sitting there bawling my eyes out…It made me feel one hundred times worse*" (PMI).[29] Impacts of these responses on PMI include fear, mistrust, trauma and PTSD.[66] PMI felt degraded and traumatised: *"I was handcuffed, led out into the street, head pushed down and into a police car. It is humiliating, the police were scaring me, neighbours witnessed the disturbance, and local youths came out to shout and jeer from the side lines"*.[43] Companions reported being interrogated by police, which exacerbated the situation,[25, 33] and that mental illness was ignored:[48, 51] *"So the victims have become the criminals"* (carer).[48]

In contrast, PMI and companions praised FRs when they used a gentle approach, avoided excessive force by engaging meaningfully with them, using de-escalation and communication techniques.[22-24, 26-28, 31, 34, 47, 52, 55-58, 60, 75-77] They valued FRs being professional, non-judgmental and compassionate, providing reassurance, expressing a wish to help, asking for permission to examine or touch PMI, and including them in decision making.[29, 30, 32, 33, 35-37, 44, 55, 76, 79] FRs assumed a non-threatening presence by maintaining eye contact, not standing with arms crossed, altering their tone and taking a low-profile approach to protect PMI privacy, and by not using police uniform or vehicles.[20, 22, 34, 57, 60, 65] FRs suggested using direct questions to avoid confusion and stress: *"the less you say, and the more you listen, the better the call will go… do you actually need to obtain that information, or is it just agitating"* (paramedic),[24] and using humour to distract and calm the PMI.[24, 27, 28, 58, 60-65]

1.2. Accountability, cautiousness and confusions about the law and procedures.

FRs expressed concerns and confusion about the law, in detaining PMI, which made decision making difficult:[67, 69, 71] *"They don't fully understand how to use the Mental Health Act…I think there's a lot of officers out there who have a lack of knowledge of what we can and can't do*" (police officer).[68] Some police officers suggested detainments are voluntarily because,[34, 59] and when PMI have crises at home it is illegal to enact S136. They described pressure from colleagues to detain PMI when they did not meet the requirements of S136,[59] and said that they often prioritised the safety of the PMI over the law in order to take them to hospital:[28, 59, 69, 71] *"I'd rather get into trouble ethically for taking somebody against their will than somebody die, and I left them there to die*" (paramedic).[69] During MHC, FRs worried about being blamed, scrutiny, losing their job and pension, and felt they had to cover themselves:[17, 31, 67, 69, 70, 71, 73] "*It's just not worth losing my job over people who don't care about themselves"* (paramedic).[73] Police officers shared their anxiety and sense of responsibility for suicide victims and their families.[70]

2. Impact of perceived legitimacy and public stigma.

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2.1. Non-legitimacy of mental illness and impact on seeking help.

Many studies reported how often mental health crises was not considered a 'real' emergency. FRs reported that responding to MHC was 'babysitting', frustrating, and a waste of time and resources, reporting that it damaged their public image.[24, 28, 49, 53, 56, 60, 61, 64, 75, 77, 96] FRs perceived PMI as selfish and taking support from other, more deserving incidents, confronting the PMI:[25, 31, 36, 43, 53, 73, 76] *"Next time you have the urge to pretend to kill yourself do not call us, call someone who can help. Because while we are pampering to your selfish needs there are others who really do need our help who are not getting it*" (paramedic).[53] These encounters often resulted the PMI feeling ashamed: "*Emotions varied and included shame and embarrassment at having wasted police time"*.[26]

Legitimacy towards PMI was particularly challenged by FRs when it involved frequent users of mental health services: "*we've had patients who attempted to commit suicide every week, just to let out their frustrations…to get our attention. I feel no empathy for such people"* (paramedic).[56] However, FRs also described PMI being frequent users of emergency services due to ineffective health systems overusing services as a 'band aid approach'.[20, 27, 34, 42, 47, 62, 65, 68, 70, 73, 80, 95] PMI were often released from hospital the same day police transported them, leading to future crises requiring police response: *"'vicious cycle' …you are exposed to that cycle so many times of taking people in, nothing happens, they get released, taking people in, nothing happens, they get released"* (police officer).[68] Additionally, FRs described forming relationships with frequent users, building trust, and knowing what to say to reassure PMI.[27, 65, 77]

Having negative experiences caused PMI and companions to be reluctant in seeking help. Being treated with force and physical restraint, or at worst the PPI being killed during an encounter with FRs, led to many regretting having called for emergency help:[24, 29, 32, 36, 42-44, 46, 74, 77, 78] *"I am afraid to call them. They have exasperated traumatic events. They have belittled me for my appearance, so I stopped wearing my helmet*." (PMI).[66] This was also demonstrated in the low standards PMI had for interactions with FRs: *"I didn't get beat up, I didn't get thrown down stairs, I didn't get punched or kicked, I didn't get tazed, that kind of stuff, so, it was good*" (PMI).[35] Family/carers described feeling guilty and blamed themselves for calling the police:[48, 51, 93] *"It was that one phone call, that one phone call destroyed everything. If I could just take it back"* (parent).[47]

2.2 Person with mental illness dehumanised, use of stereotypes and stigma.

Studies demonstrated that the stigmatisation of PMI involved fear and beliefs that PMI are unpredictable, violent, dangerous, and lack insight about their illness:[23, 27, 31, 34, 75, 81] *"They're dangerous…they can snap in a minute"* (police officer).[58] FRs believed PMI were incapable of cooperation and had an 'increased pain threshold'.[35, 40, 41, 53, 55, 65] PMI reported being treated in a dehumanised way: *"They just handcuffed me like a dog, they fired me in the back of the van like a dog and they just left me in there."* (PMI).[40] PMI were treated as if they were in control of their mental health, and accused of being manipulative and disingenuous.[43] After police learned of PMI's mental illness, PMI describe a negative change in their attitude,[35] including not listening or take PMI seriously, using stigmatising language, laughing at PMI, and being insensitive, disrespectful, and judgemental:[30, 35-37, 53, 66] *"You can tell right away that they all have the same look and that stuff."* (police officer).[94] However, many studies found that FRs condemned mental illness-associated stigma and believed PMI deserved to be treated with the same urgency and professionalism as physical injuries:[24, 28, 31, 34, 37, 59, 60, 69, 64, 71, 73, 80] *"Just because we're cops doesn't mean we're better than them. . . would you be angry with him if he had cancer?"* (police officer).[64]

3. First responder capability and skills dealing with mental health crises.

3.1 Role requirements and training: Recognising symptoms of mental illness

FRs often failed to resolve MHC, as they believed MHC response is not part of their role, they have too much responsibility, and they felt let down by inadequate MH services.[22, 24, 28, 31, 34, 48, 55, 61, 62, 65-67, 72, 83, 85-90] Due to limited hours and resources of MH services, FRs were often the only service available to deal with MHC. Police felt they have few options when hospitalization was impossible: doing nothing or arresting PMI for 'breach of the peace'.[27, 43, 51, 53, 58, 71] A confusion regarding their role and responsibilities was documented; many FRs believed they had a duty of care[22, 31, 34, 56, 68, 76, 96, 82]

and others stated that FRs should not respond to MHC, which was shared by some PMI and family/carers.[24, 59, 73, 96]

FRs felt unequipped to respond to MHC due to their lack of knowledge, resources, and time constraints:[24, 28, 34, 38, 58, 59, 61, 64, 70, 71, 73, 82, 87, 88, 89, 91, 95, 98] *"training makes it look straight forward and it's dead simple…and then you go out and it's just a different game"* (police officer).[61] Police described their past reluctance to ask PMI direct questions, thinking they would distress them, and instead asked vague questions that overwhelmed and confused PMI.[24] Carers reported that police were unprofessional and, in some cases, refused to take the PMI to the hospital.[25, 28, 29, 30, 35, 37, 46, 66, 74, 93] One officer argued that families do not understand the requirements of detainment, but others described incentives and initiatives to reduce use of S136.[17, 61] FRs described their MH training as minimal, unspecific, unhelpful, and not prioritised, resulting in poor knowledge and inability to manage MHC.[22, 24, 27, 31, 32, 55, 60, 61, 68, 71, 73, 88] Paramedics felt isolated during MHC, lacked guidelines to support their decisions and used a trial-and-error approach[24, 28, 63, 69] A lack of training promotes fear, stigma and stereotypes, putting PMI at risk of excessive force and criminalisation.[27, 29, 31, 35, 55]

PMI believed FRs need training in compassion, patience, communication skills, de-escalation techniques, building rapport, asking questions, and confidentiality.[22, 35, 52, 55, 60] After Crisis Intervention Team (CIT) training, FRs reported better understanding and identification of MHCs, and being equipped to ask direct questions and spend more time with PMI.[24, 39, 57, 58, 72, 86, 87, 89, 90, 96] Training reduced the unpredictability of MHC and FRs felt safer, more confident and patient.[22, 74, 93, 94, 96] Training normalised mental illness and increased FR empathy by challenging stigma and stereotypes, resulting fewer injuries and arrests, and more support for PMI:[39, 57, 58, 81, 83, 86, 89, 90] *"You know, after you come out of CIT training, you understand that it's an illness that's treatable and that, you know, just normal people can suffer from a mental illness*'' (police officer).[90] However, a number of FRs reported that training would be of no use without adequate mental health services.[55, 74, 83]

3.2 Interagency collaboration Interagency collaboration for crisis response

Paramedics valued police involvement because of their familiarity with the public and ability to encourage patients to go to hospital.[24, 66, 83] Police valued how collaboration can prevent arrests and challenge agency stereotypes.[24, 57, 64, 66, 85, 87] PMI and carers found initiatives such as the Street Triage response friendly, effective and safe,[7, 17, 29, 37, 54, 91] and police felt that it improved relations between paramedics and police, leading to better and faster response.[17, 54, 57, 61, 91] However, FRs also described onsite tensions, power struggles and arguments about who should take the PMI to hospital, and which behaviours posed a 'serious threat'.[24, 27, 32, 59, 83, 91] In addition, police were frustrated at paramedics' low level priority for PMI and long waiting times that caused distress.[29, 30] Sometimes police decided not to call for an ambulance.[59] One PMI reported a lack of collaboration between agencies, and that the multi-agency response was overwhelming.[29] Police also describe how agencies rejecting PMI under the influence exacerbated mental distress and left them no choice but to arrest PMI for their own safety.[41, 48, 56, 64 83] Police did not know who to call or what resources were available.[24] PMI believed that even a gentle response from police, without MHPs present, was irrelevant because as police are not qualified.[30, 32, 35, 53]

3.3 The value of personal experience.

FRs believe that personal experience of mental illness is more valuable than formal training. They described how personal experience and 'on the job training" aided their responses significantly more than formal training:[22, 24, 28, 34, 60, 68, 70, 73, 77] *"I don't think there's any textbook that can teach you"* (police officer).[70] FRs described using common sense to guide their approach: *"after 22 years I get a pretty good idea of whether someone's off their… basically mentally incapacitated or not. I don't know I would probably go on a gut feeling more than anything else*" (police officer).[70] Paramedics believed responding to MHC is too complex and that they are always learning on the job:[22, 34, 63, 65, 77] *"That's just defusing a bomb and learning how to do that, and knowing what agencies to call...I have the years of experience, but it's not what I'm trained in*" (paramedic).[24]

4. Impact of response on companions: involvement hindering vs facilitating response.

4.1 Companions ignored, not informed and impact on them not recognised.

Companions reported not being informed as to why there was an arrest and what will happen next.[26, 30-32, 35-43] They described the crisis response as intrusive, terrifying, and traumatic.[24, 30, 44, 46-49] They reported receiving no support after the crisis, which worsened their suffering: "*No one would ring me up and talk to me. Here I was, a mother in crisis because her son [was in] crisis"* (mother).[24] This lack of support for companions was also reported by FRs.[24] Failure to communicate with PMI/companions worsen the crises: *"If they had sat down with me instead and said 'okay, do you want to tell us what's been going on…it would have been easier and they wouldn't have needed the handcuffs because I wouldn't have ran"* (companion).[26]

4.2. Companion involvement: hindering response or leading to better outcomes

PMI preferred an informal crisis response involving friends and family,[29] however the interaction between FRs-PMI-companions is complex: *The family can be very instrumental in helping…sometimes the family is actually more difficult to manage than the patient"* (paramedic).[22] During a crisis, family members felt responsible for the PMI and calling the police was a last resort.30 FRs reported that family presence can complicate their response, as sometime there may be conflict between the PMI and family members.[24, 25, 27, 28] When the presence of family prevented PMI cooperation, sometimes due to shame, paramedics separated the family: *"They just don't want friends/family knowing what they're going through"* (paramedic).[67]

However, in many instances, PMI and police described family and friends' involvement as calming and encouraging:[24, 31] *"I was sort of like cajoled by my brother and my mate … they said, oh you know they wanted to take me to (the psychiatric hospital), and I said right-oh, no worries"* (PMI).[32] Police obtained information from parents about PMI, for example whether they are aggressive or cooperative, to enable an appropriate police response:[33, 34] *"When it is the first time for us with that person to be transported, the first thing we do is to contact a relative to know about what type of person he is"* (police officer).[67]

Discussion

Findings reflect the numerous factors at play during a mental health crisis in the community. The differing experiences on the quality of response from PMI and their family/carers, suggests important differences between and within first police and paramedic services. The main factors influencing response are persistent stigmatised attitudes among first responders, arbitrary training, lack of mental health services and the triadic interactions between FRs, PMI and companions present at the crisis.

Evident in the majority of studies was that whilst many first responders engaged meaningfully and empathetically to resolve crises, a significant number of interactions involved excessive force, physical restraint and stereotyping, which often humiliated and traumatised PMI and their companions.[23, 27, 29, 31, 32, 34-37, 38, 40, 42-46, 48-55, 75, 81] Previous reviews[100-102] reported FRs reasons for force, such as their duty of care, bystanders and themselves, and that PMI are likely to be resistant and would this require proportionate force.[29, 32, 35-37, 39, 40, 42-46, 48-55] FRs suggested that using force is required to gain control and central to their profession[34, 53, 57-59] and dealing with MHC was not, at least officially, a central part of their role. This lack of understanding and inadequate policies and procedures create a somewhat understandable confusion for FRs as well as fear that leads to the criminalisation of people with mentally illness.

Issues of stigmatisation and disregarding mental illness as a legitimate health condition were also revealed in the studies.[23, 27, 31, 34, 35, 40, 41, 53, 55, 65, 75, 81] This demonstrates the difficulty of eliminating stigma, unconscious bias and the unintended consequences of attitudes. New studies[47] continue to find stigmatising views amongst officers. We found that although all FRs wanted to challenge stigma[24, 31, 34, 37, 59, 60, 64, 67, 69, 71, 72, 80] there was a variation in attitudes and practice. A review[103] also

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found diverse attitudes towards PMI among FRs, for example, more experienced paramedics had more anger towards PMI and saw them as irritating and a waste of time, whereas others found that more experienced paramedics had more empathy. This suggests perhaps in order to dismantle prejudices, personal experiences of FRs, PMIs and their companions need to be incorporated in education and training, as *personal* and not necessarily *more* experience can lead to a more empathetic response.

Views from all stakeholders in this review revealed a confusion over whether FRs were the appropriate service for MHC response. Many believed they were not qualified, that MH services were responsible, and that training is futile with an inadequate health system.[24, 30, 32, 35, 53, 55, 59, 73, 86, 82, 87, 96] Previous reviews[102] also reported that improving these interactions is irrelevant when there are no support services available, which often results in 'frequent attenders', where inadequate response in emergency and in-patient settings leads to a cycle of repeated crises.[104] Stakeholders had conflicting views on inter-agency collaboration but was overall valued by FRs.[24, 55, 61, 73, 76, 85] Although guidance suggests that police 'should expect support from 'health and social care services within locally agreed timescales',[6] FRs in the majority of studies emphasised lack in resources. Both paramedics and police viewed dealing with MHC as an superfluous part of their role. Different types of training for police and paramedics could help integrate MHC response into their 'official roles' and alleviate tensions between the PMI needs versus the organisation's needs.

All stakeholders called for training to improve FRs' understanding and knowledge of MHC and to breakdown stigma and stereotypes.[22, 24, 26, 31, 32, 55, 58, 59, 63, 82, 93] FR criticism of their MH training indicates an interest and need in improving MHC response. In line with other research,[105] we found that personal experience is valued by police officers. 'On the job training' was perceived by some as more important than formal training.[22, 24, 28, 34, 60, 65, 70, 73, 77]. Evidence that colleagues sharing their personal experience[34, 53, 58, 59] can help understanding and response, as well as CIT training, [24, 39, 57, 58, 72, 86, 87, 89, 90, 96] show that better strategies that can help FRs are available. This highlights the importance of finding ways for FRs to share their practical knowledge and skills during training or team meetings.

Often a MH episode will take place in a home with other people, often family, present. In line with research of parents of children with psychosis,[106] companions described the often traumatic impact of the FR response and a lack of inclusion and communication.[24, 26, 30, 32, 35-43, 47, 48, 74] Our findings reflect that of previous studies,[1, 107] that show that family/carers experience distress, guilt, reluctance to call FRs, fear of disproportionate use of force, but also relief for having FR support. From the FR perspective, the presence of companions during the crisis complicates the response, necessitating attempts to calm the PMI whilst acknowledging family/carers distress. FRs described both the invaluable presence of companions in terms of giving information, but also how their involvement could hinder their response, e.g. when the PMI does not trust or wants the companion involved. Future research should explore how FRs navigate this complex relationships and use a compassionate and constructive/beneficial approach when third parties are involved.

Strengths and limitations

This review used a rigorous methodology, including a comprehensive literature search, clearly defined inclusion criteria and independent screening and data extraction.[54] Of the 79 included studies, 41 were published from 2015 indicating that most research on this topic is contemporary. All of the nine sub-themes were consistently reported by at least twelve studies and all included studies used first order interpretation (direct quotes from participants) strengthening the analysis. However, only studies published in English were included and the focus of the review was limited to interactions and personal experiences and did not include effectiveness studies of interventions to improve MHC response.

Conclusion

This review demonstrates the variety of experiences and complexity of responding to a MHC and the many diverse challenges FRs face. During MHCs, FRs are responding to the individual in crisis, seeking information and dealing with distressed family/carers, who can either by helpful or obstructive in resolving the crises. Future research is required into how FRs navigate this triadic relationship in the context of a MHC. All stakeholders call for improved training and this review displays instances

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where training has been effective in eliminating stigma and less use of force. FRs preference of 'on the job' training implies the importance of FRs sharing practical knowledge and future research can investigate how to do this in the hectic settings of police and ambulance stations. The belief that FRs are not qualified to respond to MHC is somewhat valid. Training and education in first response to MCH should consider these issues as well as broader issues of legitimacy and procedural/institutional barriers.

Abbreviations	Meaning
PMI	Person with lived experience of mental Illness
MH	Mental health
MHC	Mental health crisis
MHP	Mental Health Professional
S136	Section 136 of the Mental Health Act 1983
FR/FRs	First responder/s (police and paramedics)
Companions	Carers, family members or friends present during interactions

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Population	Interest	Context
- People with lived experience of a MHC - FRs: police, paramedics, firefighters, emergency medical technicians - Companions present in MHC (e.g. carers, family, friends)	The lived experiences of the response.	The interaction in community settings.

Figure 1.PICo

165x41mm (150 x 150 DPI)

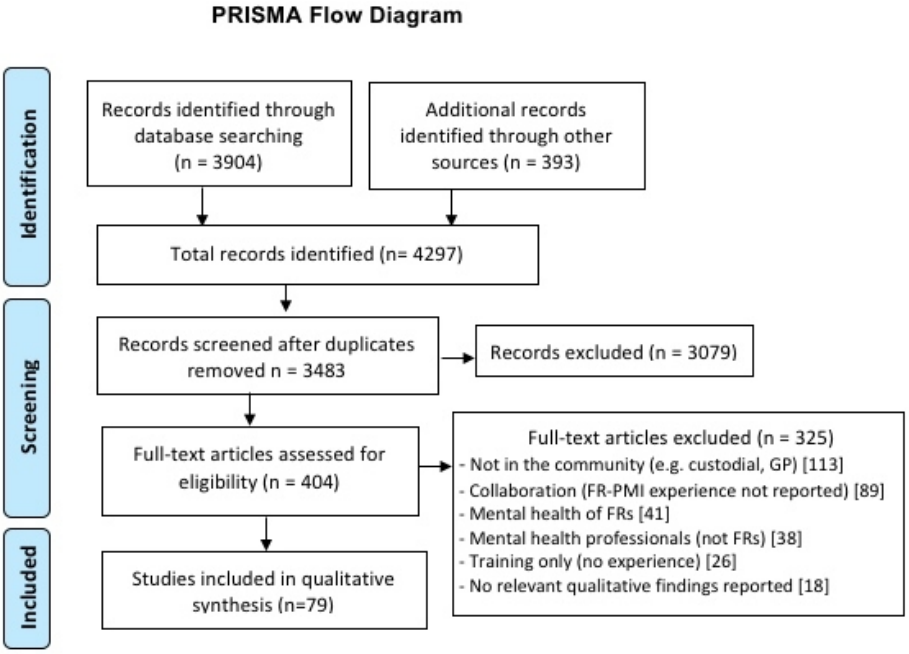


Figure 2. PRISMA Flow Diagram

215x159mm (72 x 72 DPI)

Appendix 1. Search Strategy.

Mental health crisis	1. exp mental health/ 2. exp self harm/ 3. exp Attempted Suicide / or exp Suicide/ or suicide.mp. 4. exp psychotic disorders/ or exp affective disorders, psychotic/ 5. exp mental illness/ 6.exp psychiatric disorder/ 7. (mental health emergency or mental health crisis).ab. or (mental health emergency or mental health crisis).ti. 8. 1 or 2 or 3 or 4 or 5 or 6 or 7
Non-mental health emergency responders	1. exp first responder/ 2. exp police/ 3. exp ambulance/ 4. (EMT or paramedic*).ab. or (EMT or paramedic*).ti. 5. emergency responder.ab. or emergency responder.ti. 6. (first responder or emergency responder or emergency medical technician).ab. or (first responder or emergency responder or emergency medical technician).ti. 7. (rescue service or firefighter or fire fighter).ab. or (rescue service or firefighter or fire fighter).ti. 8. 1 or 2 or 3 or 4 or 5 or 6 or 7
Qualitative design	((("semi-structured" or semistructured or unstructured or informal or "in-depth" or indepth or "face-to-face" or structured or guide) adj3 (interview* or discussion* or questionnaire*))).ti,ab. or (focus group* or qualitative or ethnograph* or fieldwork or "field work" or "key informant").ti,ab. or interviews as topic/ or focus groups/ or narration/ or qualitative research/ Filter:https://libguides.sph.uth.tmc.edu/search_filters/ovid_medline_filters

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PRISMA 2020 Checklist

Section and Topic	Item #	Checklist item	Location where item is reported
TITLE			
Title	1	Identify the report as a systematic review.	p.1
ABSTRACT			
Abstract	2	See the PRISMA 2020 for Abstracts checklist.	p.1
INTRODUCTION			
Rationale	3	Describe the rationale for the review in the context of existing knowledge.	Background p.2
Objectives	4	Provide an explicit statement of the objective(s) or question(s) the review addresses.	Background p.2
METHODS			
Eligibility criteria	5	Specify the inclusion and exclusion criteria for the review and how studies were grouped for the syntheses.	p.3
Information sources	6	Specify all databases, registers, websites, organisations, reference lists and other sources searched or consulted to identify studies. Specify the date when each source was last searched or consulted.	p.2
Search strategy	7	Present the full search strategies for all databases, registers and websites, including any filters and limits used.	p.2 and appendix 1.
Selection process	8	Specify the methods used to decide whether a study met the inclusion criteria of the review, including how many reviewers screened each record and each report retrieved, whether they worked independently, and if applicable, details of automation tools used in the process.	p.2
Data collection process	9	Specify the methods used to collect data from reports, including how many reviewers collected data from each report, whether they worked independently, any processes for obtaining or confirming data from study investigators, and if applicable, details of automation tools used in the process.	p.3
Data items	10a	List and define all outcomes for which data were sought. Specify whether all results that were compatible with each outcome domain in each study were sought (e.g. for all measures, time points, analyses), and if not, the methods used to decide which results to collect.	p.3 and Table 2.
	10b	List and define all other variables for which data were sought (e.g. participant and intervention characteristics, funding sources). Describe any assumptions made about any missing or unclear information.	p.3 and table 1.
Study risk of bias assessment	11	Specify the methods used to assess risk of bias in the included studies, including details of the tool(s) used, how many reviewers assessed each study and whether they worked independently, and if applicable, details of automation tools used in the process.	p.3- Screening, data extraction and synthesis
Effect measures	12	Specify for each outcome the effect measure(s) (e.g. risk ratio, mean difference) used in the synthesis or presentation of results.	n/a
Synthesis methods	13a	Describe the processes used to decide which studies were eligible for each synthesis (e.g. tabulating the study intervention characteristics and comparing against the planned groups for each synthesis (item #5)).	p.3 and table 2.
	13b	Describe any methods required to prepare the data for presentation or synthesis, such as handling of missing summary statistics, or data conversions.	p.2
	13c	Describe any methods used to tabulate or visually display results of individual studies and syntheses.	p.3 and table 2.
	13d	Describe any methods used to synthesize results and provide a rationale for the choice(s). If meta-analysis was performed, describe the model(s), method(s) to identify the presence and extent of statistical heterogeneity, and software package(s) used.	p.3 - Screening, data extraction and synthesis
	13e	Describe any methods used to explore possible causes of heterogeneity among study results (e.g. subgroup analysis, meta-regression).	n/a

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PRISMA 2020 Checklist

Section and Topic	Item #	Checklist item	Location where item is reported
	13f	Describe any sensitivity analyses conducted to assess robustness of the synthesized results.	n/a
Reporting bias assessment	14	Describe any methods used to assess risk of bias due to missing results in a synthesis (arising from reporting biases).	n/a
Certainty assessment	15	Describe any methods used to assess certainty (or confidence) in the body of evidence for an outcome.	n/a
RESULTS			
Study selection	16a	Describe the results of the search and selection process, from the number of records identified in the search to the number of studies included in the review, ideally using a flow diagram.	p.3 and table 1
	16b	Cite studies that might appear to meet the inclusion criteria, but which were excluded, and explain why they were excluded.	Figure 2
Study characteristics	17	Cite each included study and present its characteristics.	
Risk of bias in studies	18	Present assessments of risk of bias for each included study.	n/a
Results of individual studies	19	For all outcomes, present, for each study: (a) summary statistics for each group (where appropriate) and (b) an effect estimate and its precision (e.g. confidence/credible interval), ideally using structured tables or plots.	n/a
Results of syntheses	20a	For each synthesis, briefly summarise the characteristics and risk of bias among contributing studies.	Table 1 characteristics
	20b	Present results of all statistical syntheses conducted. If meta-analysis was done, present for each the summary estimate and its precision (e.g. confidence/credible interval) and measures of statistical heterogeneity. If comparing groups, describe the direction of the effect.	p.3 – p.7 for qualitative synthesis results.
	20c	Present results of all investigations of possible causes of heterogeneity among study results.	n/a
	20d	Present results of all sensitivity analyses conducted to assess the robustness of the synthesized results.	n/a
Reporting biases	21	Present assessments of risk of bias due to missing results (arising from reporting biases) for each synthesis assessed.	n/a
Certainty of evidence	22	Present assessments of certainty (or confidence) in the body of evidence for each outcome assessed.	n/a
DISCUSSION			
Discussion	23a	Provide a general interpretation of the results in the context of other evidence.	p.7-8
	23b	Discuss any limitations of the evidence included in the review.	p.8
	23c	Discuss any limitations of the review processes used.	p.8
	23d	Discuss implications of the results for practice, policy, and future research.	p.8
OTHER INFORMATION			
Registration and protocol	24a	Provide registration information for the review, including register name and registration number, or state that the review was not registered.	n/a. Not registered as design revised during extraction and then could not register: "Reviews that have started or completed data extraction are not eligible for inclusion in PROSPERO."
	24b	Indicate where the review protocol can be accessed, or state that a protocol was not prepared.	A protocol was not prepared.

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PRISMA 2020 Checklist

Section and Topic	Item #	Checklist item	Location where item is reported
	24c	Describe and explain any amendments to information provided at registration or in the protocol.	n/a
Support	25	Describe sources of financial or non-financial support for the review, and the role of the funders or sponsors in the review.	p.9
Competing interests	26	Declare any competing interests of review authors.	p.9
Availability of data, code and other materials	27	Report which of the following are publicly available and where they can be found: template data collection forms; data extracted from included studies; data used for all analyses; analytic code; any other materials used in the review.	p.9

From: Page MJ, McKenzie JE, Bossuyt PM, Boutron I, Hoffmann TC, Mulrow CD, et al. The PRISMA 2020 statement: an updated guideline for reporting systematic reviews. BMJ 2021;372:n71. doi: 10.1136/bmj.n71

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Subjective experiences of the response to mental health crises in the community: A qualitative systematic review.

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Abstract

Objective: To review and synthesise qualitative studies that have explored subjective experiences of people with lived experience of mental illness/crisis (MHC), their families and first responders, of their interactions) in the community.
Design: A systematic review of qualitative evidence was conducted. English language articles exploring the content of interactions and participants' experiences were included.
Data sources: MEDLINE, PsycINFO, EMBASE, CINAHL; Google scholar, SAGE journals, Science Direct, and PubMed.
Data extraction and synthesis: Two reviewers read and systematically extracted data from the included papers. Papers were appraised for methodological rigour using the Critical Appraisal Skills Programme (CASP) Qualitative Checklist. Data were thematically analysed.
Results: We identified 3483 unique records, 404 full-texts were assessed against the inclusion criteria and 79 studies were included in the qualitative synthesis. FRs identified in studies were police and ambulance staff. Main factors influencing response are persistent stigmatised attitudes among first responders, arbitrary training, and the triadic interactions between FRs, PMI and 3rd parties present at the crisis. In addition, FR personal experience of mental illness and focused training can help create a more empathetic response, however lack of resources in mental health services continues to be a barrier where 'frequent attenders' are repeatedly let down by mental health services.
Conclusion: Lack of resources in mental healthcare and rise in mental illness, suggest that FR response to MHC is inevitable. Inconsistent training, complexity of procedures and persistent stigmatisation make this a very challenging task. Improving communication with family carers and colleagues could make a difference. Broader issues of legitimacy and procedural barriers should be considered in order to reduce criminalisation and ensure an empathetic response.

Strengths and limitations of this study

- As far as we are aware, this is the first systematic review bringing together the experiences of all stakeholders involved in a mental health crisis in the community.
- We undertook rigorous searches for the available literature and assessed the quality of the studies.
- Sixty-six of the included studies were assessed to be of 'high' methodological quality (≥9 out of 10) and 13 were of 'intermediate' quality (7 or 8 out of 10).
- The majority of studies were conducted in developed countries, which could limit the generalisability of the findings.
- This review included studies published in the English language only.

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Background

Mental health-related crises (MHC) in the community are increasing. This can partly be attributed to increased transitions from institutions to community treatment,[1] while the Covid-19 pandemic also had an impact, with the National Health Service in the UK, reporting a significant increase in urgent and emergency referrals of people in crisis.[2] Police and paramedics are often the first responders (FRs) to a MHC.[3,4] In the UK 40% of police time is spent responding to a MHC[5] The World Health Organisation[6] calls for 'community actors such as police officers' to be trained in empowering people with mental illness.

However, there are concerns that FRs cannot provide expert support for people with mental illness (PMI) and report feeling ill-equipped.[7, 8] Limited training can result in inappropriate responses,[5, 9] for instance, tasers are more likely to be used during a MHC than a criminal arrest.[5] Due to limited resources the only options available to police have been force and detention.[9] Section 136 (S136) of the Mental Health Act 1983 gives police authority to detain PMI in a safe place, often a cell, which can worsen the crisis.[9,10] Concerns about police capability to make decisions about people's mental health has led to changes in legislation and policy. The Policing and Crime Act 2017 requires that police consult mental health professionals (MHPs) before using S136, use a health-based place of safety instead of a police station, and reduce the period of detention from 72 to 24 hours.[11]

Ways to improve FR response include specific training tools[12] which mainly involving de-escalation techniques for police, as well as models of triage[7] that aim to improve police officers' ability to provide effective response.[13] Triage includes Crisis Intervention Teams (CIT) where a police member is trained in mental health crisis intervention, and co-response/interagency collaborations between FR and mental health professionals[7]. The aim is to improve response and to reduce the likelihood of detention.[7] A recent review[13] of interagency collaboration models found that there is a wide range, with different kinds of agencies and services involved at various levels. Co-responder interventions are not routine practice and the majority not evaluated,[13] and high costs, logistics[14,13] and limited access to local mental health services[15] make such models difficult to implement.

Currently, evidence on FR interactions and communication practices in incidents involving people with mental ill health is not well documented.[11] Factors that impact on the interactions between FRs and PMI need to be understood better and integrated into training to improve response.[16] Significant research focuses on service development, however there is a need for research on lived experiences and interactions as they unfold in real time, exploring the perspectives of all stakeholders, including companions (family/carers/friends).[17, 18] With rises in mental distress and illness worldwide, identifying experiences of the responses to MHC from all people involved is critical. Hence, the aim of this study was to review available qualitative evidence on the subjective experiences of people with lived experience, their companions and first responders, of the FR response to mental health crises in the community.

Methods

The PICo for qualitative studies (figure 1) was used to identify the *Population* of interest, the phenomena of *Interest* and the *Context*.[19]

Figure 1. PICo

Search Strategy

A systematic search was conducted using the MEDLINE in Process (Ovid), PsycINFO (Ovid), EMBASE (Ovid) and CINAHL databases. Relevant research was also identified using Google scholar, SAGE journals, Science Direct, PubMed, Hand searching systematic reviews' reference lists, reference checking and citation chasing of included studies. A search strategy was developed using controlled vocabulary unique to each database and free-term texts including three search groups: Mental health crisis; non-mental health emergency responders, and qualitative design (appendix 1).

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Selection criteria

Retrieved articles from the inception for each database to November 2020 were included, if they directly related to all three search groups. Articles published in English were included because there was no multilingual researcher in the team, and finite resources. Studies reporting perspectives/experiences, involving adults (≥18 years) and using a qualitative approach, including mixed methods with a clear qualitative aspect. We only included qualitative articles to allow for understanding of the experiences of all stakeholders involved in these interactions. Studies with exclusively quantitative data, non-adult participants, that reported solely on mental health professionals, and the MHC did not occur in the community, were excluded.

Screening, data extraction and synthesis

Two reviewers conducted an initial screening of all titles and abstracts against the inclusion criteria to identify relevant papers. Duplicates were removed. Retrieved records were first screened on title and abstract. Reviewers screened the relevant full papers, and studies were independently assessed for eligibility. Disparities were resolved in weekly meetings before proceeding to the next stage. The Critical Appraisal Skills Programme (CASP) Qualitative Checklist[20] was used to appraise the methodological quality of the articles. The extraction and analysis of the qualitative studies was iterative[21] and was reviewed in weekly analytic meetings. Findings of each study, including participant quotations and author interpretations, were examined, extracted, and assigned to descriptive codes in NVIVO12. Inductive thematic synthesis was used where findings of primary studies were coded line by line and similar characteristics were then grouped together and given a label to describe the content. These descriptive labels (codes) were then grouped into “sub-themes”, which were then developed into descriptive themes. This process was conducted by two authors (PX, CT) and in 2 further analytic meetings JD.

Patient and Public Involvement

Exploratory work in 2019 was conducted with:
1. Carers and people with lived experience (South West of England)
2. Five Ambulance Service staff
3. Devon & Cornwall Police lead and for Enhanced Communication

FR response was described by carer and PMI as “hit and miss”, “dismissive” but also positive “they [police] always looked after me”. FRs described the challenges they face and lack of, or difficulties in following prescriptive training. This led to development of aims of this review and the focus on real-life experiences.

Results

The Electronic and hand searches identified 4297 studies (Figure 2, PRISMA flow diagram). Seventy-nine studies, including 3 unpublished, were included in the review (Table 1). These studies used semi-structured qualitative interviews (n=43), focus groups (n=4) and mixed methods (n=32). Studies explored the perspectives of the police (n=35), paramedics (n=13), PMI (n=15), carers or family members (n=8) and multiple stakeholders (n=8). No other FR perspectives, e.g., firefighters, were found. The studies were conducted in the UK (n=22), USA (n=22), Australia (n=15), Canada (n=10), The Netherlands (n=2), New Zealand, France, Sweden, Ireland, Ghana, Slovenia, Portugal, and Guyana (each n=1). All scored 7 or higher on the CASP Checklist (see table 1), indicating medium to high quality; no exclusions were made on this basis. Reasons for lower scores were: a) not reporting on the relationship between researchers and participants, b) limited reporting on ethical issues and c) not detailed reporting on the analysis.

Figure 2. PRISMA Flow Diagram of included and excluded studies.

Table 1. Summary of included studies.

Author & year	Country	Perspective	Study focus	Study design and methods	Sample size (qualitative)	CASP score
Adio *et al* 2020[22]	USA	Paramedics	Views of paramedics towards frequent emergency department users and underlying causes.	Mixed-method design. Survey & interviews.	16 Community Integrated Health	9 out of 10

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					Program paramedics.	
Baker and Pillinger 2020[23]	USA	Family members	To understand how police interactions with PMIs can lead to deaths and families' experiences.	Semi-structured interviews.	16 family members of a citizen who died after police contact.	9 out of 10
Baker and Pillinger 2020[24]	UK	Family members	The police role in providing a de facto service for people undergoing mental health crises.	Semi-structured interviews.	12 family members of a citizen who died after police contact.	8 out of 10
Bendelow *et al* 2019[25]	UK	PMI & companions	To investigate the complexities underlying the high rates of s136.	Mixed-methods. Interviews & written accounts.	37 people with lived experience of detention.	8 out of 10
Bohrman *et al* 2018[26]	USA	Police	Police officers assessment for mental illnesses and variation by location.	Semi-structured interviews.	15 officers.	9 out of 10
Boscarato *et al* 2014[27]	Australia	PMI	Consumers' experiences with formal crisis-response mechanisms - police-only and joint responses.	Exploratory study. Semi-structured interviews.	11 mental health consumers.	9 out of 10
Bradbury *et al* 2017[28]	Australia	PMI, carers & Service providers	To explore the lived experience of involuntary transport under the mental health act.	Cross-sectional, qualitative research design. Interviews.	16 participants: 6 consumers, 4 carers 6 service providers.	9 out of 10
Brennan *et al* 2016[29]	Australia	Carers	Carers' experiences of mental health crises and responses provided by police and mental health services.	Semi-structured interviews.	9 carers for someone with a mental illness.	9 out of 10
Brink *et al* 2011[30]	Canada	PMI	How people with mental illness perceive and interact with the police.	Participatory Action Research. Focus groups, interview, survey.	60 people with lived experience.	9 out of 10
Booty *et al* 2020[31]	USA	Police	Officer attitudes pre- and post-CIT training.	Mixed-methods study. Surveys, group discussions, interviews.	14 Central District patrol officers.	7 out of 10
Callender *et al* 2021[17]	UK	Police	To compare the processes, experiences and perceptions of Mental health street triage.	Semi-structured interviews.	27 police and health service staff.	9 out of 10
Canada *et al* 2012[32]	USA	Police	Impact of CIT on police officers' response to mental illness calls.	Qualitative research design. Survey and phone interviews.	216 officers.	9 out of 10
Cheng *et al* 2020[33]	USA	Family caregivers	Needs of family caregivers for early psychosis.	Semi-structured focus group interviews.	20 family caregivers.	9 out of 10
Copeland & Heilemann 2008[34]	USA	Family caregivers	Mothers understanding of violence of their mentally ill, adult children.	Grounded theory methodology, open-ended interviews.	8 mothers.	9 out of 10
Daggenvoorde *et al* 2018[35]	Netherlands	PMI & families	Lived experiences of patients with a psychotic or bipolar disorder and their families of a mobile crisis team.	Phenomenological study design. Interviews.	10 patients and10 family members.	9 out of 10
Davey *et al* 2019[36]	New Zealand	Police	E-Learning for police with regard to people experiencing mental distress.	Semi-structured telephone interviews.	24 police staff.	9 out of 10

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Dejean *et al* 2016[37]	Canada	Paramedics	Paramedics views on appropriate versus inappropriate ambulance use.	Constructivist grounded theory methodology. Interviews.	19 paramedics.	9 out of 10
Dougall 2014[38]	Canada	Police	Lived experiences of frontline police personnel of a mid-sized police service.	Exploratory study, heuristic phenomenology. Semi-structured interviews	7 police officers, 4 communicators and 3 administrators.	10 out of 10
Dyer *et al* 2015[39]	UK	Police	Implementation of Cleveland Police's pilot Street Triage (ST) service.	Exploratory design. Interviews and retrospective analysis of case notes.	16 stakeholders, case notes, and administrative data.	7 out of 10
Erdner & Piskator 2013[40]	Sweden	Police	Police officers' experiences of committing individuals with mental illness to the hospital for treatment.	Semi-structured interviews. Qualitative content analysis.	7 police officers.	9 out of 10
Ferguson *et al* 2019[41]	Australia	PMI	Men's experiences of ambulance care for mental health and/or AOD problems.	Exploratory qualitative approach. Semi-structured interviews.	30 men.	9 out of 10
Ford-Jones 2020[42]	Canada	Paramedics	Mental health and psychosocial calls encountered by paramedics in the community.	Case study. Interviews.	10 paramedic management/educators and 5 directors/physicians.	9 out of 10
Ford-Jones & Daly 2020[43]	Canada	Paramedics	Paramedic service practices that are connecting patients to mental health and psychosocial programming.	Qualitative interviews and observations.	36 paramedics, 5 educators, 5 directors/physicians.	9 out of 10
Gerson *et al* 2009[44]	USA	Family members	Experiences of families seeking treatment for young people with recent-onset psychosis.	Qualitative open-ended interviews.	14 particpants-13 interviews (one pair)	9 out of 10
Gibbs & Haas 2020[45]	Australia	PMI	Experiences of autistic people who had interacted with police.	Mixed-methods design. Questionnaire and interviews.	12 autistic adults	9 out of 10
Girard *et al* 2014[46]	France	PMI & Police	Mental health team and police responding to persons who are homeless with serious psychiatric disorders.	Mixed qualitative and quantitative methods. Focus group. Minutes of meetings.	40 PMI (interactions). Focus groups: 12 police officers.	9 out of 10
Godfredson *et al* 2011[47]	Australia	Police	Contact between the police and people experiencing mental illness.	Survey. Thematic analyses of open-ended responses.	3,534 police officers.	8 out of 10
Goodall *et al* 2019[48]	UK	PMI	Process of being detained under S136 of the Mental Health Act (1983, 2007).	Semi-structured interviews.	15 people detained under Section 136 of the Mental Health Act.	9 out of 10
Green 1997[49]	USA	Police	Police involvement with suspects who have a mental illness.	Mixed methods. Qualitative structured & semi-structured interviews.	11 police officers	7 out of 10
Gregory & Thompson 2013[50]	UK	PMI	Personal experiences of a service user through a mental health crisis.	Auto-ethnography and interviews. Reflective narrative.	1 service user.	7 out of 10
Haas & Gibbs 2020[51]	Australia	PMI and parents	Impact that autistic characteristics on interactions with police.	Interviews, content analysis.	12 autistic adults and 19 parents.	9 out of 10
Hanafi *et al* 2008[52]	USA	Police	To evaluate the effectiveness of Crisis Intervention Team (CIT) training for police officers.	Qualitative focus group study.	25 officers.	9 out of 10

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Herrington & Pope 2014[53]	Australia	Police, NSW Health & NGO representatives	Evaluate the success of the MHIT.	A multi-phased, mixed methods design. Interviews, observations	56 interview participants.	7 out of 10
Holmes 2017[54]	UK	PMI	Missing people with mental health issues and police response.	Qualitative semi-structured interview & free text narratives.	45 formerly missing adults with mental health issues.	7 out of 10
Keefe *et al* 2020[55]	USA	Paramedics	Paramedics' experiences and perceptions regarding behavioral health emergencies.	Qualitative in-depth interviews.	25 paramedics.	8 out of 10
Krayer *et al* 2018[56]	UK	Police; statutory & third sector organisations	Perceptions about joint working between mental health, social care and police services with regard to anti-social behaviour.	Multi-method sequential qualitative study. Police logs; interviews; focus groups.	55 statutory and third sector organisations; 60 cases-narrative police logs	9 out of 10
Lamb & Tarpey 2019[57]	UK	Police	UK police officers views on their experience of working with people with mental health difficulties.	Social constructionist perspective. Interviews.	10 police officers	7 out of 10
Lane 2019[58]	UK	Police	Forum members discourses on individuals experiencing mental health difficulties.	Discursive psychology and critical discourse analysis.	75 threads – discussion forum for members of the police force	9 out of 10
Livingston *et al* 2014[59]	Canada	PMI	Experiences of people with mental illness of their interactions with the police.	Semi-structured interviews.	60 people with mental illness.	8 out of 10
Mahmuda *et al* 2020[60]	Canada	PMI (Frequent users)	Frequent users' experiences regarding EMS.	A Grounded theory approach. Semi-structured interviews.	10 participants.	8 out of 10
Marsden *et al* 2020[61]	UK	Police	UK police officers' experiences of their involvement in mental healthcare.	Qualitative methods. Semi-structured interviews and a vignette.	15 police officers.	9 out of 10
McCann *et al* 2018[62]	Australia	Paramedic	Paramedics' experience of caring for patients with non-medical emergency- related mental health and/or AOD problems.	Framework Method. Semi-structured interviews.	73 paramedics.	10 out of 10
McGuinness *et al* 2013[63]	Ireland	PMI	Impact of involuntary hospital admission to a mental health centre and illuminate their lived experience of involuntary admission.	Interpretative Phenomenological Analysis. Semi-structured interviews.	6 participants.	10 out of 10
McKenna *et al* 2015[64]	Australia	Police, carers, paramedics, MH staff	Perceptions of stakeholders on behavioural escalation service utilization of people in mental health crisis.	Exploratory research design. Interviews	17 people (advisors, ED staff, police, ambulance officers).	9 out of 10
McLean & Marshall 2010[65]	UK	Police	Challenges when policing mental ill-health and use of mental health triage in England and Wales.	Interpretative phenomenological analysis. Semi-structured interviews.	9 police officers.	9 out of 10
Morgan & Paterson 2019[66]	UK	Police	Police officers views of working with people with mental health	Human rights framework. Semi-structured interviews.	7 police officers.	7 out of 10

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			difficulties and their experience of training to equip them for this.			
Myers *et al* 2019[67]	USA	PMI	Real-time decision-making between first hospital admission for early psychosis and treatment engagement or drop-out.	Prospective, longitudinal, ethnographic study. Open-ended, person-centred interviews	37 people who experienced involuntary admission.	8 out of 10
Ogloff *et al* 2013[68]	Australia	Police	Association among mental disorder, police shootings, and other injuries, and police interactions with victims of crime.	Mixed-methods. semi-structured interviews.	25 police officers.	9 out of 10
Olasoji *et al* 2017[69]	Australia	Carers	Experiences of carers as they access mental health services and as they engage with service/health providers?	A qualitative descriptive approach. focus groups	19 Carers of patients (i.e., family members)	10 out of 10
Osafo *et al* 2017[70]	Ghana	Police	Views of the police on persons who attempt suicide and the law criminalizing the act.	Qualitative approach. Semi-structured in-depth interviews	18 officers	9 out of 10
Oxburgh *et al* 2016[71]	UK	Police	Perceptions of police officers regarding MD suspects, support provided to MD suspects, and current police training in MD.	Grounded Theory. Questionnaire consisting of a mixture of open and probing questions	35 police officers	9 out of 10
Prener & Lincoln 2015[72]	USA	Paramedic	Experiences, beliefs, and attitudes toward what EMS providers call “psych calls,”	Semi-structured observational method and interviews.	4 x12hr observations Interviews: 4 EMTs & 16 paramedics.	9 out of 10
Railey *et al* 2020[73]	USA	Police, carer and PMI	LEOs’ knowledge of ASD, interactions between LEOs and individuals with ASD, and training needs.	Constructivist grounded theory approach (interviews and questionnaires).	6 adults with ASD, 5 caregivers and 6 law enforcement officers	10 out of 10
Rant & Bregar 2014[74]	Slovenia	Paramedic	Paramedics experience suicidal patients.	Qualitative case study, semi-structured interviews.	10 paramedics.	9 out of 10
Rees *et al* 2017[75] AND Rees *et al* 2018[76]	UK	Paramedic	Paramedics' perceptions and experiences of caring for people who self-harm.	Evolved Grounded Theory Methodology. Semi structured interviews.	11 paramedics.	10 out of 10
Roberts & Henderson 2009[77]	Australia	Paramedic	Paramedics use of resources to support practice and their role when dealing with patients displaying mental illness.	Mixed methods. Focus groups and survey (open questions).	74 survey responders. Focus groups: 20 paramedics.	9 out of 10
Rolfe *et al* 2020[78]	UK	Paramedic	Paramedics managing patients experiencing mental health issues.	Qualitative. Observations and interviews.	21 paramedics and 20 patients with mental illness.	10 out of 10
Ross *et al* 2020[79]	Australia	Police	Effectiveness of the Masterplan in reducing suicides.	Mixed-methods design. Interviews.	8 police officers.	9 out of 10
Salerno & Schuller 2019[80]	Canada	Police	Interactions between adults with ASD and the police and (2) exploring how individuals with ASD perceive these experiences.	Community-engaged research strategy and integrating both qualitative and quantitative research methods.	35 adults with ASD.	9 out of 10
Schulenberg 2016[81]	Canada	Police	Dynamics of police and PMI encounters vs non-PMI.	Observation and sequential mixed methods research design. Field notes.	Data on 606 citizens (74 ride-alongs).	8 out of 10

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Shaban 2011[82]	Australia	Paramedics	Paramedics accounting for their JDM with respect to mental illness in the emergency care setting.	Ethnographic methods. Semi-structured interviews.	6 paramedics.	9 out of 10
Soares & Pinto da Costa 2019[83]	Portugal	Police	Police officers' interactions in compulsory admissions and in the delivery of involuntary mental health treatment.	Qualitative research. In-depth semi-structured interviews.	10 police officers.	9 out of 10
Sondhi *et al* 2018[84]	UK	PMI	Perceptions of the process for people who have been detained under section 136 of the Mental Health Act 1983.	Qualitative design. Semi-structured interviews.	58 people with lived experience detained under section 136 and 4 carers.	9 out of 10
Spence & Millott 2016[85]	UK	Police	To explore police negotiators' perceptions of preparation and coping responses related to suicide.	Qualitative design. Semi-structured interviews.	16 police negotiators.	9 out of 10
Stokoe & Sikveland 2020[86]	UK	Police	Methods negotiators use to engage persons in crisis and initiate and maintain productive sequences of talk.	Qualitative design. Conversation analysis.	14 negotiations, 31hrs of audio-recordings.	9 out of 10
Tully & Smith 2015[87]	USA	Police	Officer perception of preparedness after receiving Crisis Intervention Team training.	Mixed-method design. semi-structured interviews	8 officers.	9 out of 10
van Steden 2020[88]	Netherlands	Police	Officers and nurses perceptions of disorderly and confused people.	Qualitative design. informal conversations, interviews, focus groups, group meetings.	5 policy makers, 17 community police officers and 3 district nurses.	9 out of 10
Wallace 2019[89]	Guyana	Police	Police officers' attitude, response and interaction with people with mental illness.	Mixed-method design. Open-ended questions of survey.	9 participants responded to open-ended questions.	9 out of 10
Warrington 2019[90]	UK	People with suicidality	Repeated Section 136 detentions.	Mixed-method design. In-depth interviews	6 participants.	9 out of 10
Watson *et al* 2008[91]	USA	PMI	Dimensions of how police encounters are experienced by consumers.	Qualitative design. In-depth semi-structured interviews	20 persons with mental illness.	9 out of 10
Watson & Wood 2017[92]	USA	Police	Examination of the Crisis Intervention Team model.	Mixed-method design. In-depth semi-structured interviews.	21 police officers.	9 out of 10
Wells & Schafer 2006[93]	USA	Police	Police officer perceptions of their contacts with the mentally ill and examine outcomes of an innovative police training program.	Mixed-method design. Open-ended survey items.	126 police officer respondents.	9 out of 10
White *et al* 2019[94]	UK	PMI	Experiences with law enforcement services, emergency departments and psychiatric services.	Author personal experience- narrative.	One personal account.	9 out of 10
Wood & Beierschmitt 2014[95]	USA	Police	Opportunities for police enhanced upstream engagement.	Mixed-method design. Focus groups & interviews.	22 police officers.	9 out of 10
Wood *et al* 2020[96]	USA	Police	Officer perspectives on the unmet needs of individuals and	Qualitative design. Field notes describing observat	57 officers.	7 out of 10

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			their families and the ways in which the mental health and social system environment constrain officers' abilities to be responsive to them.	ions and ride-along interviews.		
Wood & Watson 2017[97]	USA	Police	The nature of encounters police have with persons affected by mental illness and the ways in which such encounters are resolved by police.	Qualitative design. Observations.	31 ride-alongs. 53 officers observed.	7 out of 10
Young *et al* 2019[98]	Canada	Parent Caregiver	Experiences of parent caregivers for adult children with schizophrenia.	Qualitative design. Interviews.	12 parent participants	7 out of 10

Table 2 presents the themes and subthemes from the thematic synthesis of the included studies, which all contained first-order themes.

Table 2. Themes and subthemes.

Theme	Subthemes	Studies exploring each sub-theme
Acknowledging versus criminalising mental illness	Criminalisation and use of force vs recognition of behaviour relating to mental ill health.	22-24, 26, 32, 34-35, 41, 45, 51, 53-54, 59, 63, 66, 81-83, 87, 90-97. (Canada, USA, UK, Australia, Ireland, Portugal)
	Accountability, cautiousness and confusion over procedure.	17, 62-63, 65-67, 79, 82-83, 85, 93, 95. (Australia, Ireland, Portugal, UK, USA)
Legitimacy and stigma	Non-legitimacy of mental illness and impact on seeking help.	23, 29, 45, 50, 53, 59, 62, 64-66, 79, 81-87, 89, 90-91, 93-95, 97. (Australia, Canada, Portugal, UK, USA)
	Stigma and use of stereotypes.	17, 23, 29, 34-35, 40, 51, 58-60, 62-63, 66, 71, 81-87 92, 98. (Australia, Canada, Netherlands, Portugal, Sweden, UK, USA)
First responder capability and skills dealing with mental health crises	Training: impact and requirements.	32, 35, 41-42, 50, 52, 56, 59, 62, 64-66, 70, 79, 81-87, 89, 93-96, 98. (Australia, Canada, Ghana, Guyana, Netherlands, Portugal, UK, USA)
	Interagency collaboration.	22, 25-27, 34-36, 46, 53, 59, 63, 65-66, 81, 84, 89, 91, 93-94, 96-97. (Australia, Canada, France, Guyana, Netherlands, New Zealand, UK, USA)
	The value of personal experience.	50, 57, 62, 66, 74, 79, 83, 87, 93-94, 97 (Australia, Portugal, Slovenia, UK, USA)
Impact of response on companions: involvement hindering vs facilitating response	Companions ignored, not informed and impact on them not recognised.	23-25, 29, 33-35, 91, 93-98 (Australia, Netherlands USA, UK)
	Companion involvement: hindering response vs leading to better outcomes.	35, 45, 83-86, 94-95, 97, 98. (Australia Canada, USA, UK, Portugal, Netherlands)

1. Acknowledging versus criminalising mental illness and procedural concerns.

1.1 Criminalisation and use of force vs recognition of behaviour relating to mental ill health.

Companions and PMI described distress, fear, confusion, humiliation and trauma caused by a large police presence, disproportionate use of S136, involuntary transport, and physical restraint: [23-24, 35, 53, 63, 90-93, 95-97] *"You're confined in this tiny, small box and you go, 'what's happening to you?' They don't*

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speak to you, and the only time they do, they're barking orders at you" (PMI).[54] FRs were sometimes perceived as threatening: *"I said I am going to call 911, and he said: 'no don't, if you call 911 they are going to kill me"* (companion).[23] In one study, paramedics describe disliking physical restraint and excessive force, believing their presence alone can be calming.[87] In contrast, police described their "right to use force",[63] arguing that force and restraint were *"most helpful because they immediately make the situation unpleasant for the individual"*.[32] Police felt victimized when criticized for using force.[32, 66, 83, 87]

PMI and their family/carers described instances where crises were not resolved. Time delays, use of force and FRs' threatening presence increased their distress:[26, 35, 91, 95-97] *"[Six police officers] were standing there with their capsicum spray…I was just sitting there bawling my eyes out…It made me feel one hundred times worse*" (PMI).[97] Impacts of these responses on PMI include fear, mistrust, trauma and PTSD.[80] PMI felt degraded and traumatised: *"I was handcuffed, led out into the street, head pushed down and into a police car. It is humiliating, the police were scaring me, neighbours witnessed the disturbance, and local youths came out to shout and jeer from the side lines"*.[94] Companions reported being interrogated by police, which exacerbated the situation,[33,] and that mental illness was ignored:[22] *"So the victims have become the criminals"* (carer).[23]

In contrast, PMI and companions praised FRs when they used a gentle approach, avoided excessive force by engaging meaningfully with them, using de-escalation and communication techniques.[32, 37, 41, 59, 83-86, 91-97] They valued FRs being professional, non-judgmental and compassionate, providing reassurance, expressing a wish to help, asking for permission to examine or touch the PMI, and including them in decision making.[29, 35, 45, 59, 65, 86, 95-97] FRs assumed a non-threatening presence by maintaining eye contact, not standing with arms crossed, altering their tone and taking a low-profile approach to protect PMI privacy, and by not using police uniform or vehicles.[50, 63, 83, 87, 93-94] FRs suggested using direct questions to avoid confusion and stress: *"the less you say, and the more you listen, the better the call will go… do you actually need to obtain that information, or is it just agitating"* (paramedic),[94] and using humour to distract and calm the PMI. [32, 59, 81-82, 87, 89, 93-94, 97]

1.2. Accountability, cautiousness and confusion over procedure.

FRs expressed concerns and confusion about the law, in detaining PMI, which made decision making difficult: [62-63, 66, 79, 82-83, 85, 93, 95] *"They don't fully understand how to use the Mental Health Act…I think there's a lot of officers out there who have a lack of knowledge of what we can and can't do*" (police officer).[66] Some police officers suggested detainments are voluntarily because,[66, 83] and when PMI have crises at home it is illegal to enact S136. They described pressure from colleagues to detain PMI when they did not meet the requirements of S136,[66] and said that they often prioritised the safety of the PMI over the law in order to take them to hospital:[66, 82, 93, 95] *"I'd rather get into trouble ethically for taking somebody against their will than somebody die, and I left them there to die*" (paramedic).[93] During MHC, FRs worried about being blamed, scrutiny, losing their job and pension, and felt they had to cover themselves:[17, 32, 62, 79, 85, 93, 95] "*It's just not worth losing my job over people who don't care about themselves"* (paramedic).[62] Police officers shared their anxiety and sense of responsibility for suicide victims and their families.[79]

2. Impact of perceived legitimacy and public stigma.

2.1. Non-legitimacy of mental illness and impact on seeking help.

Many studies reported how often mental health crises were not considered a 'real' emergency. FRs reported that responding to MHC was 'babysitting', frustrating, and a waste of time and resources, reporting that it damaged their public image.[56, 59, 64, 81, 86-87, 93-95] FRs perceived PMI as selfish and taking support from other, more deserving incidents, confronting the PMI:[62, 65, 84-85, 87, 91, 94] *"Next time you have the urge to pretend to kill yourself do not call us, call someone who can help. Because while*

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we are pampering to your selfish needs there are others who really do need our help who are not getting it" (paramedic).[87] These encounters often resulted in the PMI feeling ashamed: "*Emotions varied and included shame and embarrassment at having wasted police time"*.[91]

Legitimacy towards PMI was particularly challenged by FRs when it involved frequent users of mental health services: "*we've had patients who attempted to commit suicide every week, just to let out their frustrations…to get our attention. I feel no empathy for such people"* (paramedic).[87] However, FRs also described PMI being frequent users of emergency services due to ineffective health systems overusing services as a 'band aid approach'.[50, 62, 65-66, 82-83, 86, 90, 93, 96] PMI were often released from hospital the same day police transported them, leading to future crises requiring police response: *"vicious cycle…you are exposed to that cycle so many times of taking people in, nothing happens, they get released, taking people in, nothing happens, they get released"* (police officer).[66] Additionally, FRs described forming relationships with frequent users, building trust, and knowing what to say to reassure PMI.[89, 93, 97]

Negative interactions with FRs caused PMI and companions to be reluctant in seeking help. Being treated with force and physical restraint, or at worst the PPI being killed during an encounter with FRs, led to many regretting having called for emergency help:[29, 79, 82, 84, 86, 90, 93-95, 97] *"I am afraid to call them. They have exasperated traumatic events. They have belittled me for my appearance, so I stopped wearing my helmet*." (PMI).[82] This was also demonstrated in the low standards PMI had for interactions with FRs: *"I didn't get beat up, I didn't get thrown down stairs, I didn't get punched or kicked, I didn't get tazed, that kind of stuff, so, it was good*" (PMI).[35] Family/carers described feeling guilty and blamed themselves for calling the police:[23, 45, 83] *"It was that one phone call, that one phone call destroyed everything. If I could just take it back"* (parent).[23]

2.2 Person with mental illness dehumanised, use of stereotypes and stigma.

Studies demonstrated that the stigmatisation of PMI involved fear and beliefs that PMI are unpredictable, violent, dangerous, and lack insight about their illness.[71, 82-83, 85-86, 98] Some FRs also believed PMI were incapable of cooperation and had an 'increased pain threshold'.[29, 63, 84, 58, 59, 92] PMI reported being treated in a dehumanised way: *"They just handcuffed me like a dog, they fired me in the back of the van like a dog and they just left me in there."* (PMI).[63] PMI were treated as if they were in control of their mental health, and accused of being manipulative and disingenuous.[94] After police learned of PMI's mental illness, PMI describe a negative change in their attitude,[35] including not listening or take PMI seriously, using stigmatising language, laughing at PMI, and being insensitive, disrespectful, and judgemental:[35, 82, 87, 96] *"You can tell right away that they all have the same look and that stuff."* (police officer).[89] However, many studies found that FRs condemned mental illness-associated stigma and believed PMI deserved to be treated with the same urgency and professionalism as physical injuries:[62, 66, 81-83, 85, 87, 93-96] *"Just because we're cops doesn't mean we're better than them...would you be angry with him if he had cancer?"* (police officer).[81]

3. First responder capability and skills dealing with mental health crises.

3.1 Role requirements and training: Recognising symptoms of mental illness

FRs often failed to resolve MHC, as they believed MHC response is not part of their role, they have too much responsibility, and they felt let down by inadequate MH services.[50, 56, 66, 79, 81, 83-87, 93-95] Due to limited hours and resources of MH services, FRs were often the only service available to deal with MHC. Police felt they have few options when hospitalization was impossible: doing nothing or arresting PMI for 'breach of the peace'.[32, 64, 82, 87, 94, 97] A confusion regarding their role and responsibilities was documented; many FRs believed they had a duty of care[50, 56, 65-66, 83, 85, 87] whilst others stated they should not respond to MHC, which was shared by some PMI and family/carers.[56, 62, 66, 94]

FRs felt unequipped to respond to MHC due to their lack of knowledge, resources, and time constraints:[41-42, 50, 53, 59, 62-63, 79, 81-83, 87-88, 90, 94-95] "*training makes it look straight forward and it's dead simple…and then you go out and it's just a different game"* (police officer).[59] Police described their past reluctance to ask PMI direct questions, thinking they would distress them, and instead asked vague questions that overwhelmed and confused PMI.[94] Carers reported that police were unprofessional and, in some cases, refused to take the PMI to the hospital.[35, 45, 82-83, 86, 93, 96-97] One officer argued that families do not understand the requirements of detainment, but others described

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incentives and initiatives to reduce use of S136.[32, 66] FRs described their MH training as minimal, unspecific, unhelpful, and not prioritised, resulting in poor knowledge and inability to manage MHC.[50, 59, 62, 66, 79, 82, 85, 87, 89, 94-95] Paramedics felt isolated during MHC, lacked guidelines to support their decisions and used a trial-and-error approach.[82, 93-95] A lack of training promotes fear, stigma and stereotypes, putting PMI at risk of excessive force and criminalisation. [35, 59, 85, 89, 97]
PMI believed FRs need training in compassion, patience, communication skills, de-escalation techniques, building rapport, asking questions, and confidentiality.[35, 50, 59, 87, 91] After Crisis Intervention Team (CIT) training, FRs reported better understanding and identification of MHCs, and being equipped to ask direct questions and spend more time with PMI.[32, 53, 63, 81, 84-85, 87, 94] Training reduced the unpredictability of MHC and FRs felt safer, more confident and patient.[37, 69, 32, 36, 56] Training normalised mental illness and increased FR empathy by challenging stigma and stereotypes, resulting fewer injuries and arrests, and more support for PMI:[32, 39, 52-53, 63, 81, 84-85, 87, 89] *"You know, after you come out of CIT training, you understand that it's an illness that's treatable and that, you know, just normal people can suffer from a mental illness*" (police officer).[52] However, a number of FRs reported that training would be of no use without adequate mental health services.[84, 87]

3.2 Interagency collaboration for crisis response

Paramedics valued police involvement because of their familiarity with the public and ability to encourage patients to go to hospital.[65, 84, 94] Police valued how collaboration can prevent arrests and challenge agency stereotypes.[59, 65, 84, 93-94] PMI and carers found initiatives such as the Street Triage response friendly, effective and safe,[53, 91, 96-97] and police felt that it improved relations between paramedics and police, leading to better and faster response.[32, 53, 59, 63, 91] However, FRs also described onsite tensions, power struggles and arguments about who should take the PMI to hospital, and which behaviours posed a 'serious threat'.[53, 66, 85, 89, 94-95] In addition, police were frustrated at paramedics' low level priority for PMI and long waiting times that caused distress.[26, 34] Sometimes police decided not to call for an ambulance.[66] One PMI reported a lack of collaboration between agencies, and that the multi-agency response was overwhelming.[97] Police also describe how agencies rejecting PMI under the influence exacerbated mental distress and left them no choice but to arrest PMI for their own safety.[81, 84, 91, 93, 96] Police did not know who to call or what resources were available.[50] PMI believed that even a gentle response from police, without MHPs present, was irrelevant because police are not qualified.[35, 59, 95]

3.3 The value of personal experience.

FRs believe that personal experience of mental illness is more valuable than formal training. They described how personal experience and 'on the job training" aided their responses significantly more than formal training: [50, 62, 66, 79, 83, 87, 93-94, 97] *"I don't think there's any textbook that can teach you"* (police officer).[79] FRs described using common sense to guide their approach: *"after 22 years I get a pretty good idea of whether someone's off their… basically mentally incapacitated or not. I don't know I would probably go on a gut feeling more than anything else*" (police officer).[93] Paramedics believed responding to MHC is too complex and that they are always learning on the job. [66, 79, 83, 87, 94]

4. Impact of response on companions: involvement hindering vs facilitating response.

4.1 Companions ignored, not informed and impact on them not recognised.

Companions reported not being informed as to why there was an arrest and what will happen next.[29, 35, 91, 94-95] They described the crisis response as intrusive, terrifying, and traumatic.[35, 93-98] They reported receiving no support after the crisis, which worsened their suffering: "*No one would ring me up and talk to me. Here I was, a mother in crisis because her son [was in] crisis"* (mother).[29] This lack of support for companions was also reported by FRs.[94] Failure to communicate with PMI/companions worsen the crises: *"If they had sat down with me instead and said 'okay, do you want to tell us what's been going on…it would have been easier and they wouldn't have needed the handcuffs because I wouldn't have ran"* (companion).[91]

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4.2. Companion involvement: hindering response or leading to better outcomes

PMI preferred an informal crisis response involving friends and family,[97] however the interaction between FRs-PMI-companions is complex: *The family can be very instrumental in helping…sometimes the family is actually more difficult to manage than the patient"* (paramedic).[94] During a crisis, family members felt responsible for the PMI and calling the police was a last resort.[35] FRs reported that family presence can complicate their response, as sometime there may be conflict between the PMI and family members.[83, 94, 97] When the presence of family prevented PMI cooperation, sometimes due to shame, paramedics separated the family: *"They just don't want friends/family knowing what they're going through"* (paramedic).[95]

However, in many instances, PMI and police described family and friends' involvement as calming and encouraging:[85, 94] *"I was sort of like cajoled by my brother and my mate … they said, oh you know they wanted to take me to (the psychiatric hospital), and I said right-oh, no worries"* (PMI).[95] Police obtained information from parents about PMI, for example whether they are aggressive or cooperative, to enable an appropriate police response:[45, 83] *"When it is the first time for us with that person to be transported, the first thing we do is to contact a relative to know about what type of person he is"* (police officer).[95]

Discussion

This review of the qualitative aimed to identify and synthesise findings from published studies on the subjective experiences of stakeholders involved in incidents/crises which involve people with mental ill health. Findings reflect the variety of experiences and complexity of responding to mental health crisis in the community. Recognising mental illness as a legitimate condition and responding with empathy rather than force and stigmatising attitudes was valued by PMI and their companions. To achieve this, all stakeholders called for further training. FRs highlighted issues regarding the law, procedures, and accountability which may constrain the response. Despite the inevitability of their involvement in these incidents, many FRs do accept they should have a role in managing mental illness in the community. Personal experience of mental illness can make a difference in the response as well as collaboration between first responder services and mental health services. Finally, the role of 3rd parties present in the interaction, such as family, friends and carers, was recognised by all stakeholders, which adds another level of complexity to these interactions.

Evident in the majority of studies was that whilst many first responders engaged meaningfully and empathetically to resolve crises, a significant number of interactions involved excessive force, physical restraint and stereotyping, which often humiliated and traumatised PMI and their companions.[22-24, 26, 32, 34-35, 51, 59, 81-83, 87, 90-97] Previous reviews [99-101] reported FRs' reasons for force, such as their duty of care, bystanders and themselves, and that PMI are likely to be resistant and would this require proportionate force. FRs suggested that using force is required to gain control and central to their profession, and dealing with MHC was not, at least officially, a central part of their role[49-66, 83]. However, the role of first responders in managing mental ill health in the community is not only vital but likely to increase due the Covid-19 pandemic,[102] the subsequent economic crisis, as well as climate change, which disproportionally affects vulnerable people and can be the cause for mental illness.[103] Similarly, although interagency collaboration models where mental health professionals are located in police stations or police cars, can improve response,[18] most of these models are not evaluated, and high costs, logistics and limited access to local mental health services make such interventions difficult to implement.[104] Lack of understanding and inadequate policies and procedures create a somewhat understandable confusion for FRs as well as fear that leads to the criminalisation of people with mental illness.

Issues of stigmatisation and disregarding mental illness as a legitimate health condition were also revealed in the studies.[17, 23, 29, 34-35, 40, 51, 58-60, 62-63, 66, 71, 81-87 92, 98] This demonstrates the difficulty of

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eliminating stigma, unconscious bias and the unintended consequences of attitudes. New studies[67] continue to find stigmatising views amongst officers. We found that although all FRs wanted to challenge stigma[62, 66, 81-83, 85, 87, 93-96] there was a variation in attitudes and practice. A review[105] also found diverse attitudes towards PMI among FRs, for example, more experienced paramedics had more anger towards PMI and saw them as irritating and a waste of time, whereas others found that more experienced paramedics had more empathy. This suggests, perhaps that in order to dismantle prejudices, personal experiences of FRs, PMIs and their companions need to be incorporated in education and training, as *personal* and not necessarily *more* experience can lead to a more empathetic response.

Views from all stakeholders in this review revealed a confusion over whether FRs were the appropriate service for MHC response. Many believed they were not qualified, that MH services were responsible, and that training is futile with an inadequate health system.[34, 84, 87] Previous reviews[101, 106] also reported that improving these interactions is irrelevant when there are no support services available, which often results in 'frequent attenders', where inadequate response in emergency and in-patient settings leads to a cycle of repeated crises. Stakeholders had conflicting views on inter-agency collaboration but was overall valued by FRs.[59, 65, 84, 93-94] Although guidance suggests that police 'should expect support from' health and social care services within locally agreed timescales',[6] FRs in the majority of studies emphasised lack in resources. Both paramedics and police viewed dealing with MHC as a superfluous part of their role. Different types of training for police and paramedics could help integrate MHC response into their 'official roles' and alleviate tensions between the PMI needs versus the organisation's needs.

All stakeholders called for training to improve FRs' understanding and knowledge of MHC and to breakdown stigma and stereotypes.[32, 39, 52-53, 63, 81, 84-85, 87, 89] FR criticism of their MH training indicates an interest and need in improving MHC response. In line with other research,[107] we found that personal experience is valued by police officers. 'On the job training' was perceived by some as more important than formal training.[50, 62, 66, 79, 83, 87, 93-94, 97] Evidence that colleagues sharing their personal experience can help understanding and response, as well as CIT training,[31, 52] shows that better strategies that can help FRs are available. This highlights the importance of finding ways for FRs to share their practical knowledge and skills during training or team meetings.

Often a MH episode will take place in a home with other people, often family, present. In line with research of parents of children with psychosis,[108] companions described the, often traumatic, impact of the FR response and a lack of inclusion and communication.[41, 54, 34, 27, 29-94, 96, 37, 69] Our findings reflect that of previous studies,[109] that show that family/carers experience distress, guilt, reluctance to call FRs, fear of disproportionate use of force, but also relief for having FR support. From the FR perspective, the presence of companions during the crisis complicates the response, necessitating attempts to calm the PMI whilst acknowledging family/carers distress. FRs described both the invaluable presence of companions in terms of giving information, but also how their involvement could hinder their response, e.g., when the PMI does not trust or wants the companion involved. Future research should explore how FRs navigate these complex relationships and use a compassionate and constructive/beneficial approach when third parties are involved.

Strengths and limitations

This review used a rigorous methodology, including a comprehensive literature search, clearly defined inclusion criteria and independent screening and data extraction. Of the 79 included studies, 41 were published from 2015 indicating that most research on this topic is contemporary. All of the nine sub-themes were consistently reported by at least twelve studies and all included studies used first order interpretation (direct quotes from participants) strengthening the analysis. However, only studies published in English were included and the focus of the review was limited to interactions and personal experiences and did not include effectiveness studies of interventions to improve MHC response. This study did not explore specific mental health problems. Further research could investigate whether first responders' response to people with different diagnoses varies and how. Differences between countries was not systematically explored. Despite potential significant differences in policing and mental health services in the countries where included studies were conducted, analytic patterns of experiences (themes and subthemes) were found *across* different countries. However, the majority of these studies were in developed countries and studies not

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reported in English were not included. Further research could explore if there is variation in the response, by country with regards culture, or regulations.

Conclusion
This review demonstrates the variety of experiences and complexity of responding to a MHC and the many diverse challenges FRs face. During MHCs, FRs are responding to the individual in crisis, seeking information and dealing with distressed family/carers, who can either by helpful or obstructive in resolving the crises. Future research is required into how FRs navigate this triadic relationship in the context of a MHC. All stakeholders call for improved training and this review displays instances where training has been effective in eliminating stigma and less use of force. FRs preference of 'on the job' training implies the importance of FRs sharing practical knowledge and future research can investigate how to do this in the hectic settings of police and ambulance stations. The belief that FRs are not qualified to respond to MHC is somewhat valid. Training and education in first response to MCH should consider these issues as well as broader issues of legitimacy and procedural/institutional barriers.

Abbreviations	Meaning
PMI	Person with lived experience of mental Illness
MH	Mental health
MHC	Mental health crisis
MHP	Mental Health Professional
S136	Section 136 of the Mental Health Act 1983
FR/FRs	First responder/s (police and paramedics)
Companions	Carers, family members or friends present during interactions

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Population	Interest	Context
- People with lived experience of a MHC - FRs: police, paramedics, firefighters, emergency medical technicians - Companions present in MHC (e.g. carers, family, friends)	The lived experiences of the response.	The interaction in community settings.

Figure 1.PICo

165x41mm (150 x 150 DPI)

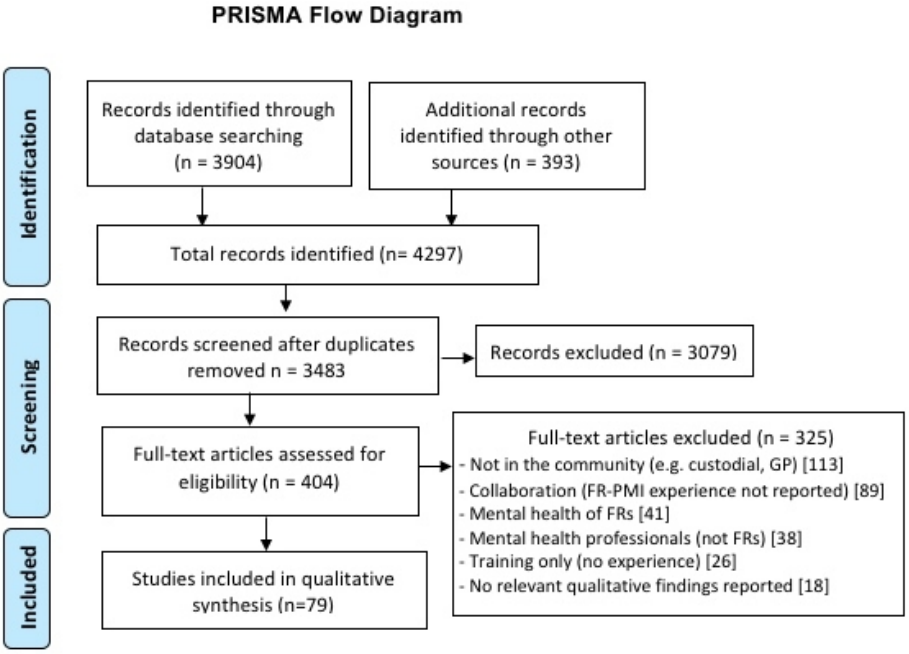


Figure 2. PRISMA Flow Diagram

215x159mm (72 x 72 DPI)

Appendix 1. Search Strategy.

Mental health crisis	1. exp mental health/ 2. exp self harm/ 3. exp Attempted Suicide / or exp Suicide/ or suicide.mp. 4. exp psychotic disorders/ or exp affective disorders, psychotic/ 5. exp mental illness/ 6.exp psychiatric disorder/ 7. (mental health emergency or mental health crisis).ab. or (mental health emergency or mental health crisis).ti. 8. 1 or 2 or 3 or 4 or 5 or 6 or 7
Non-mental health emergency responders	1. exp first responder/ 2. exp police/ 3. exp ambulance/ 4. (EMT or paramedic*).ab. or (EMT or paramedic*).ti. 5. emergency responder.ab. or emergency responder.ti. 6. (first responder or emergency responder or emergency medical technician).ab. or (first responder or emergency responder or emergency medical technician).ti. 7. (rescue service or firefighter or fire fighter).ab. or (rescue service or firefighter or fire fighter).ti. 8. 1 or 2 or 3 or 4 or 5 or 6 or 7
Qualitative design	((("semi-structured" or semistructured or unstructured or informal or "in-depth" or indepth or "face-to-face" or structured or guide) adj3 (interview* or discussion* or questionnaire*))).ti,ab. or (focus group* or qualitative or ethnograph* or fieldwork or "field work" or "key informant").ti,ab. or interviews as topic/ or focus groups/ or narration/ or qualitative research/ Filter:https://libguides.sph.uth.tmc.edu/search_filters/ovid_medline_filters

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PRISMA 2020 Checklist

Section and Topic	Item #	Checklist item	Location where item is reported
TITLE			
Title	1	Identify the report as a systematic review.	p.1
ABSTRACT			
Abstract	2	See the PRISMA 2020 for Abstracts checklist.	p.1
INTRODUCTION			
Rationale	3	Describe the rationale for the review in the context of existing knowledge.	Background p.2
Objectives	4	Provide an explicit statement of the objective(s) or question(s) the review addresses.	Background p.2
METHODS			
Eligibility criteria	5	Specify the inclusion and exclusion criteria for the review and how studies were grouped for the syntheses.	p.3
Information sources	6	Specify all databases, registers, websites, organisations, reference lists and other sources searched or consulted to identify studies. Specify the date when each source was last searched or consulted.	p.2
Search strategy	7	Present the full search strategies for all databases, registers and websites, including any filters and limits used.	p.2 and appendix 1.
Selection process	8	Specify the methods used to decide whether a study met the inclusion criteria of the review, including how many reviewers screened each record and each report retrieved, whether they worked independently, and if applicable, details of automation tools used in the process.	p.2
Data collection process	9	Specify the methods used to collect data from reports, including how many reviewers collected data from each report, whether they worked independently, any processes for obtaining or confirming data from study investigators, and if applicable, details of automation tools used in the process.	p.3
Data items	10a	List and define all outcomes for which data were sought. Specify whether all results that were compatible with each outcome domain in each study were sought (e.g. for all measures, time points, analyses), and if not, the methods used to decide which results to collect.	p.3 and Table 2.
	10b	List and define all other variables for which data were sought (e.g. participant and intervention characteristics, funding sources). Describe any assumptions made about any missing or unclear information.	p.3 and table 1.
Study risk of bias assessment	11	Specify the methods used to assess risk of bias in the included studies, including details of the tool(s) used, how many reviewers assessed each study and whether they worked independently, and if applicable, details of automation tools used in the process.	p.3- Screening, data extraction and synthesis
Effect measures	12	Specify for each outcome the effect measure(s) (e.g. risk ratio, mean difference) used in the synthesis or presentation of results.	n/a
Synthesis methods	13a	Describe the processes used to decide which studies were eligible for each synthesis (e.g. tabulating the study intervention characteristics and comparing against the planned groups for each synthesis (item #5)).	p.3 and table 2.
	13b	Describe any methods required to prepare the data for presentation or synthesis, such as handling of missing summary statistics, or data conversions.	p.2
	13c	Describe any methods used to tabulate or visually display results of individual studies and syntheses.	p.3 and table 2.
	13d	Describe any methods used to synthesize results and provide a rationale for the choice(s). If meta-analysis was performed, describe the model(s), method(s) to identify the presence and extent of statistical heterogeneity, and software package(s) used.	p.3 - Screening, data extraction and synthesis
	13e	Describe any methods used to explore possible causes of heterogeneity among study results (e.g. subgroup analysis, meta-regression).	n/a

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PRISMA 2020 Checklist

Section and Topic	Item #	Checklist item	Location where item is reported
	13f	Describe any sensitivity analyses conducted to assess robustness of the synthesized results.	n/a
Reporting bias assessment	14	Describe any methods used to assess risk of bias due to missing results in a synthesis (arising from reporting biases).	n/a
Certainty assessment	15	Describe any methods used to assess certainty (or confidence) in the body of evidence for an outcome.	n/a
RESULTS			
Study selection	16a	Describe the results of the search and selection process, from the number of records identified in the search to the number of studies included in the review, ideally using a flow diagram.	p.3 and table 1
	16b	Cite studies that might appear to meet the inclusion criteria, but which were excluded, and explain why they were excluded.	Figure 2
Study characteristics	17	Cite each included study and present its characteristics.	
Risk of bias in studies	18	Present assessments of risk of bias for each included study.	n/a
Results of individual studies	19	For all outcomes, present, for each study: (a) summary statistics for each group (where appropriate) and (b) an effect estimate and its precision (e.g. confidence/credible interval), ideally using structured tables or plots.	n/a
Results of syntheses	20a	For each synthesis, briefly summarise the characteristics and risk of bias among contributing studies.	Table 1 characteristics
	20b	Present results of all statistical syntheses conducted. If meta-analysis was done, present for each the summary estimate and its precision (e.g. confidence/credible interval) and measures of statistical heterogeneity. If comparing groups, describe the direction of the effect.	p.3 – p.7 for qualitative synthesis results.
	20c	Present results of all investigations of possible causes of heterogeneity among study results.	n/a
	20d	Present results of all sensitivity analyses conducted to assess the robustness of the synthesized results.	n/a
Reporting biases	21	Present assessments of risk of bias due to missing results (arising from reporting biases) for each synthesis assessed.	n/a
Certainty of evidence	22	Present assessments of certainty (or confidence) in the body of evidence for each outcome assessed.	n/a
DISCUSSION			
Discussion	23a	Provide a general interpretation of the results in the context of other evidence.	p.7-8
	23b	Discuss any limitations of the evidence included in the review.	p.8
	23c	Discuss any limitations of the review processes used.	p.8
	23d	Discuss implications of the results for practice, policy, and future research.	p.8
OTHER INFORMATION			
Registration and protocol	24a	Provide registration information for the review, including register name and registration number, or state that the review was not registered.	n/a. Not registered as design revised during extraction and then could not register: "Reviews that have started or completed data extraction are not eligible for inclusion in PROSPERO."
	24b	Indicate where the review protocol can be accessed, or state that a protocol was not prepared.	A protocol was not prepared.

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PRISMA 2020 Checklist

Section and Topic	Item #	Checklist item	Location where item is reported
	24c	Describe and explain any amendments to information provided at registration or in the protocol.	n/a
Support	25	Describe sources of financial or non-financial support for the review, and the role of the funders or sponsors in the review.	p.9
Competing interests	26	Declare any competing interests of review authors.	p.9
Availability of data, code and other materials	27	Report which of the following are publicly available and where they can be found: template data collection forms; data extracted from included studies; data used for all analyses; analytic code; any other materials used in the review.	p.9

From: Page MJ, McKenzie JE, Bossuyt PM, Boutron I, Hoffmann TC, Mulrow CD, et al. The PRISMA 2020 statement: an updated guideline for reporting systematic reviews. BMJ 2021;372:n71. doi: 10.1136/bmj.n71

For more information, visit: http://www.prisma-statement.org/

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